A RIGHT TO CARE, A RIGHT TO WELFARE

A STUDY OF THE *KALAIGNAR MAHALIR URIMAI THITTAM*

By Prabha Kotiswaran, Gale Andrew, Jannet Farida Jacob, Narbadeshwar Mishra, Priyadarshini S, and Rebecca Hannah



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EXECUTIVE SUMMARY

Since 2020, several states have rolled out unconditional cash transfers to women at breakneck speed in the wake of assembly elections. This phenomenon has generated debates amongst feminists, particularly feminist economists, on the impact of cash transfers on the female labour force participation rate and the high levels of unpaid domestic and care work that Indian women undertake. Meanwhile, experts on welfare characterise this steady expansion of welfare schemes, and direct benefit transfers in particular, as a form of techno-patrimonialism. Yet Tamil Nadu's *Kalaignar Mahalir Urimai Thittam* ('KMUT') is the first unconditional cash transfer scheme to articulate the payment as recognition of women's unpaid domestic and care work, which paves the way for the realisation of the United Nations Sustainable Development Goal 5.4.

The Laws of Social Reproduction project seeks to intervene in these debates by testing assumptions made by governments, feminists, and welfare experts alike on the desirability of unconditional cash transfers and their impact on women's empowerment. We undertook a mixed-methods study of the *Kalaignar Mahalir Urimai Thittam* in September 2024, 12 months after its launch, assessing the impact of the scheme across six key outcome indicators from a gendered perspective.

Our study found that the scheme is well targeted and implemented. We found significant improvement in women's financial inclusion, financial well-being, and access to financial infrastructure alongside modest improvements in food consumption, intra-household bargaining power, and political empowerment. While the scheme did not disincentivise education or entry into paid employment, its impact on female labour force participation, unpaid work burdens, and violence were mixed.

We recommend the focus of KMUT be sharpened to build upon its recognition of women's unpaid work, to realise all aspects of SDG 5.4, and truly achieve women's empowerment. To achieve this, we suggest changes in programme design alongside the pursuit of a gender transformative welfare regime as well as structural changes that include improving women's prospects for decent work, strengthening public health infrastructure, regulating microfinance institutions, and addressing the social costs of male alcoholism.

ACKNOWLEDGEMENTS

Work on this project would not have been possible without the support of several organisations and individuals. We would like to thank Populous Empowerment Network (PEN), especially Anant Mani, Sathish Kumar Vedachalam, and Dr. Devika Rani, who facilitated meetings with government officials, helped us access data relevant for the study, and assisted us in securing permission to conduct the study in Tamil Nadu. We are also grateful for the support of the Special Programme Implementation Department of the Tamil Nadu state government in granting us permission to conduct the study and providing data on the operation of the KMUT scheme. We also thank the many local government officials who helped us during the survey process.

The research was undertaken with the assistance of DAI Research & Advisory Services, which facilitated both the quantitative survey and qualitative post-survey focus group discussions. We would especially like to thank Sitaram Mukherjee, Shreya Padiyar, and Ragavi Jaganathan, who led and coordinated this undertaking, as well as the numerous field supervisors and enumerators who helped execute the survey in a comprehensive and efficient manner.

In addition, we thank Sister Josephine Amala Valarmathi, Coordinator of the National Domestic Workers Movement, who helped arrange focus group discussions with domestic workers in Chennai as well as Sujata Mody, president of *Urimai Kural's Pen Thozhilalar Sangam*, who helped the team meet domestic workers, construction workers, and garment workers for focus group discussions. We thank V. Geetha and A.S. Kumari, Chairperson of Tamil Nadu State Commission for Women and DMK Women's Wing Joint Secretary for their insights on the KMUT scheme.

Finally, above all, we would like to thank the respondents of the survey for giving us their time and sharing their experiences with us. We hope this report helps bring their experiences to light and does their stories justice.

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CHAPTER 1

INTRODUCTION

In the past five years, India has witnessed a rapid expansion of its welfare regime for women. Since 2020, several states have rolled out unconditional cash transfer schemes (UCTs) to socio-economically vulnerable women in India. It is now routine for promises of such UCTs to be made during every state assembly election, with 11 states now having implemented UCTs to around 11.3 crore[1] women.[2]

The fiscal and electoral impact of these schemes have generated intense debate on their purpose and viability; with the UCTs being accused of being 'freebies for votes'; of undercutting investments in jobs, infrastructure, health, and education; besides cultivating a passive *labhbharti* class of beneficiaries (Aiyar, 2023; 2025). However, they also represent a significant shift in welfare policy given their unconditional nature and the scale of their distribution to socio-economically vulnerable women.

In fact, international experiences with cash transfer schemes targeted at women have demonstrated their effectiveness in improving health, nutritional, and educational outcomes for women and children (UKDFID, 2011; Bastagli, 2016; Light *et al.*, 2024). But a majority of such schemes are conditional in nature, in that the transfer requires the completion of specific tasks aimed at achieving health, nutritional, and educational outcomes, all of which fall to women to perform and have been criticised by feminists for reinforcing the gendered division of labour (Molyneux, 2008; Cookson, 2018). India's cash transfer schemes are therefore unique in the scale of coverage as well as their unconditional nature. While there are legitimate concerns about the design and implementation of such schemes, they also have the unique potential to improve, if not transform, gender relations in the country.

In the Indian context, these transfers need to be assessed against the backdrop of the low female labour force participation rate, which can be attributed to the disproportionate burden of unpaid domestic and care work ('UDCW') on women in the household, among other factors (Singh and Pattanaik, 2020; Deshpande and Kabeer, 2024). Interestingly, some states have presented UCTs as offering recognition of women's UDCW, signifying a step towards realising Sustainable Development Goal 5.4 which calls on states to "recognize and value unpaid care and domestic work through the provision of public services, infrastructure and social protection policies and the promotion of shared responsibility within the household and the family as nationally appropriate" (United Nations Department of Economic and Social Affairs, 2025).

The *Kalaignar Mahalir Urimai Thittam* ('KMUT') in Tamil Nadu is one such scheme. In the run up to the Tamil Nadu state assembly elections in 2021, Kamal Haasan, the leader of a new political party, *Makkal Needhi Maiam*, promised that his party would provide "salaries for housewives" (IANS, 2020). While it was controversial at the start, other

1 113 million.

2 For more details on these schemes, see Kotiswaran *et al.* (2025).

political parties soon followed suit with similar promises (PTI, 2021). When the *Dravida Munnetra Kazhagam* party came to power in 2021, they fashioned the state's unconditional cash transfer scheme for women, the KMUT, whereby women from poorer socio-economic classes would be eligible for a monthly payment of Rs. 1000[3] (Kumar, 2023). Currently 1.14 crore[4] women are beneficiaries of this flagship welfare scheme of the Tamil Nadu government (The Hindu Bureau, 2025a).

Unlike most such cash transfer schemes, the KMUT scheme explicitly identifies the transfer as a recognition of women's UDCW and a redressal of the gender pay gap. In debates surrounding such transfers, concerns were expressed that such transfers will lead to a reduction in women's education or paid employment; or increase their burden of unpaid domestic and care work within the home (Khaitan, 2021). But one could argue that recognising UDCW is the first step towards their reduction and redistribution. Notably, the KMUT scheme has been introduced in Tamil Nadu, a state that has both a higher female labour participation rate than the national average and a social-justice-oriented model of development, also referred to as the Dravidian model, which seeks to achieve inclusive growth (Kalaiyarasan and Vijayabaskar, 2021). Tamil Nadu therefore represents an ideal testing ground for such unconditional cash transfer schemes, given the state's capacity to build and implement a robust welfare regime.

The Laws of Social Reproduction project undertook an empirical study of the KMUT's impact a year into its rollout, setting out to ascertain women's own views on UCTs and assess both the intended and unintended consequences of the scheme. The study represented an opportunity to critically analyse the role of UCTs in recognising UDCW as well as their potential to further help reduce and redistribute UDCW with a view to realising SDG 5.4 and thereby empower women. Such a study is crucial given that cash transfer programs in other parts of the world have not foregrounded women's experiences or their empowerment (Molyneux, 2008; Molyneux *et al.*, 2016). In this way, we hope this study can offer valuable insights on the potential and limitations of UCTs in other Indian states.

3 Approximately 12 USD.

4 11.4 million.

CHAPTER 2

OVERVIEW OF SCHEME

The KMUT scheme was rolled out by the Tamil Nadu state government in September 2023, offering Rs. 1000[5] per month to the female head of eligible households (see Text Box 1). At the launch of the scheme as well as at public events that followed, the Chief Minister and his cabinet Ministers have reiterated that the transfer is an entitlement for women's hard work (The Hindu Bureau, 2023a).

The scheme notification highlights that, "Women play a significant role in society by investing countless hours in both domestic responsibilities and external work. They contribute to men's accomplishments, ensure the education and well-being of children, and support the overall functioning of society" (Government of Tamil Nadu, 2023a). These detailed guidelines present the KMUT as "recognition to the efforts of women who work tirelessly throughout their lives for their families... (and) to improve the livelihood of women, enhance their standard of living and pave the way for them to live with self-respect in society".

TEXT BOX 1: ELIGIBILITY CRITERIA

The cash transfer will be provided to the female head of the eligible household who:

- is at least 21 years of age
- holds a ration card
- is a resident of Tamil Nadu
- belongs to a household that:
  - earns an annual income below 2.5 lakhs[6]
  - holds less than 5 acres of wetland or 10 acres of dryland
  - consumes less than 3600 units of electricity per year for domestic use

The scheme also includes households headed by single, unmarried women, widows, and "transgenders" as well as separated and divorced women.

**Note: Only one woman in each household receives the benefit.*

5 Approximately 12 USD.

6 Approximately 3000 USD.

Beneficiaries will be excluded if their family members:

- Pay Income Tax or Goods and Services Tax.
- Are employees or pensioners of the state and central government, public sector undertakings, boards, local bodies, or cooperatives.
- Are elected people's representatives other than village ward members.
- Have four-wheel vehicles for their own use.
    - In June 2025, the exclusion criteria was modified such that women in households that owned four-wheeler vehicles obtained through government subsidies could avail of KMUT (Government of Tamil Nadu, 2025).
- Receive social security pensions such as old age pension, widow pension, unorganised workers' welfare pension and families receiving pension from the government.
    - After the scheme was rolled out, these exclusions were reversed so that otherwise eligible women from families with members receiving pensions across various schemes including the Indira Gandhi Old Age Pension Scheme, Old Age Pension from the Unorganized Welfare Boards and the Chief Minister's Farmers Security Scheme could now apply under the scheme (Government of Tamil Nadu, 2023b).
    - In June 2025, the exclusion criteria were further modified so that eligible women in households where another member of the household was receiving pension as a widow including as a deserted widow, deserted woman, a poor unmarried woman above the age of 50 years or a special time scale government employee could avail of KMUT (Government of Tamil Nadu, 2025).

Families that include members who suffer from a range of listed medical conditions and those receiving a maintenance allowance from the Department of Welfare of Differently Abled Persons are exempt from these exclusion criteria, provided they fulfil the remaining eligibility criteria.

The KMUT scheme includes a State Monitoring Committee under the chairmanship of the Chief Secretary comprising the Development Commissioner and the Secretaries to Government across eight departments. This Committee is to periodically review the progress of the scheme and guide the District Collectors in the implementation of the scheme.

Applicants under the scheme whose applications are rejected receive an SMS with the reason for rejection and have 30 days to file an appeal against the said rejection (The Hindu Bureau, 2023b). In 2023, across three phases, 5.50 lakh (550,000) applications were received under the scheme, with 1.52 lakh (152,000) applications rejected for various reasons (The Hindu Bureau, 2023c). By February 2025, from a total of 1.96 crore[7] ration card holders in the state, the scheme reached 1.14 crore[8] beneficiaries (Tamil Nadu Civil Supplies Corporation, 2020; The Hindu Bureau, 2025a).

7 19.6 million.

8 11.4 million.

CHAPTER 3

METHODOLOGY

To study the impact of KMUT, we adopted a mixed methods approach that followed international development models for the impact evaluation of social protection programmes. In particular, we were guided by the extensive experience of impact evaluations by the Transfer Project, which undertook national evaluations of social cash transfer programmes across eight countries in Africa (Davis *et al.*, 2016).

We began with pre-survey focus group discussions (FGDs) with 121 women across two districts, followed by a survey of 2,221 women in six districts and finally held post-survey FGDs with 167 women across three districts.

PRE-SURVEY FGDS

We conducted 12 pre-survey FGDs with beneficiaries of the scheme in February 2024, six months into the rollout of the KMUT. We met a total of 121 participants across five villages in Coimbatore district and six villages in Tiruvannamalai district. These districts represented geographic spread and varied levels of development within the state, with the participants from various caste groups and including a mix of home makers and women engaged in paid employment.



Photo 1: Pre-Survey FGD

The initial FGDs sought to understand the paid and unpaid work undertaken by beneficiaries, including their own understanding of the distribution and recognition of unpaid work; their perception of and experience with the KMUT scheme, including access to the scheme and control over money received; the scheme's impact on household dynamics; and finally, their recommendations for improvement of the scheme. A discussion of these FGDs can be found in *Women's Work, Never Done, Now Paid: Assessing Tamil Nadu's Urimai Thogai Scheme* (Kotiswaran, 2025).

QUANTITATIVE SURVEY

Based on this qualitative research, we built a survey tool and conducted a quantitative survey in September and October 2024, one year into the roll-out of the KMUT, that compared the impact of the scheme on beneficiaries with an identified control group across six key outcome indicators. Treatment and control districts, taluks, as well as villages, were all systematically identified using government-provided data on the uptake of the KMUT scheme.

A significant challenge in this exercise was the lack of baseline data on the socio-economic condition of beneficiaries and non-beneficiaries prior to the introduction of the KMUT scheme. Hence, a key assumption underlying our analysis is that members of the control and treatment groups were socio-economically on par with each other prior to the scheme's roll out. Furthermore, beneficiaries of the scheme were selected into the scheme (treatment) based on eligibility criteria and not on a random basis, making it difficult to identify identical groups among non-beneficiaries and potentially creating a selection bias.



Photo 2: Survey Interview with Respondent

SAMPLING STRATEGY

We obtained district-level data on KMUT beneficiaries from government sources alongside the total number of women eligible to vote in each district. Additionally, we acquired district-level exponential growth rates from official records, allowing us to accurately estimate the latest district population levels. We computed the KMUT uptake rate as the ratio of the total number of district-level beneficiaries to the total number of eligible women

voters. Using this metric, we classified treatment districts as those with the highest KMUT uptake above the mean, that is, Dharmapuri, Nagapattinam, and Virudhunagar; and control districts as those with the lowest uptake below the mean, that is, Coimbatore, Vellore, and Kancheepuram.

After selecting districts, we obtained taluk-level beneficiary data and corresponding population figures from government records and applied the same procedure to compute the KMUT uptake rate at the taluk level and then classify taluks into treatment and control groups. We then followed the same process at the village level, estimating the KMUT uptake rate based on the number of beneficiaries relative to the local population. We selected treatment villages with the highest KMUT uptake and control villages with the lowest uptake.

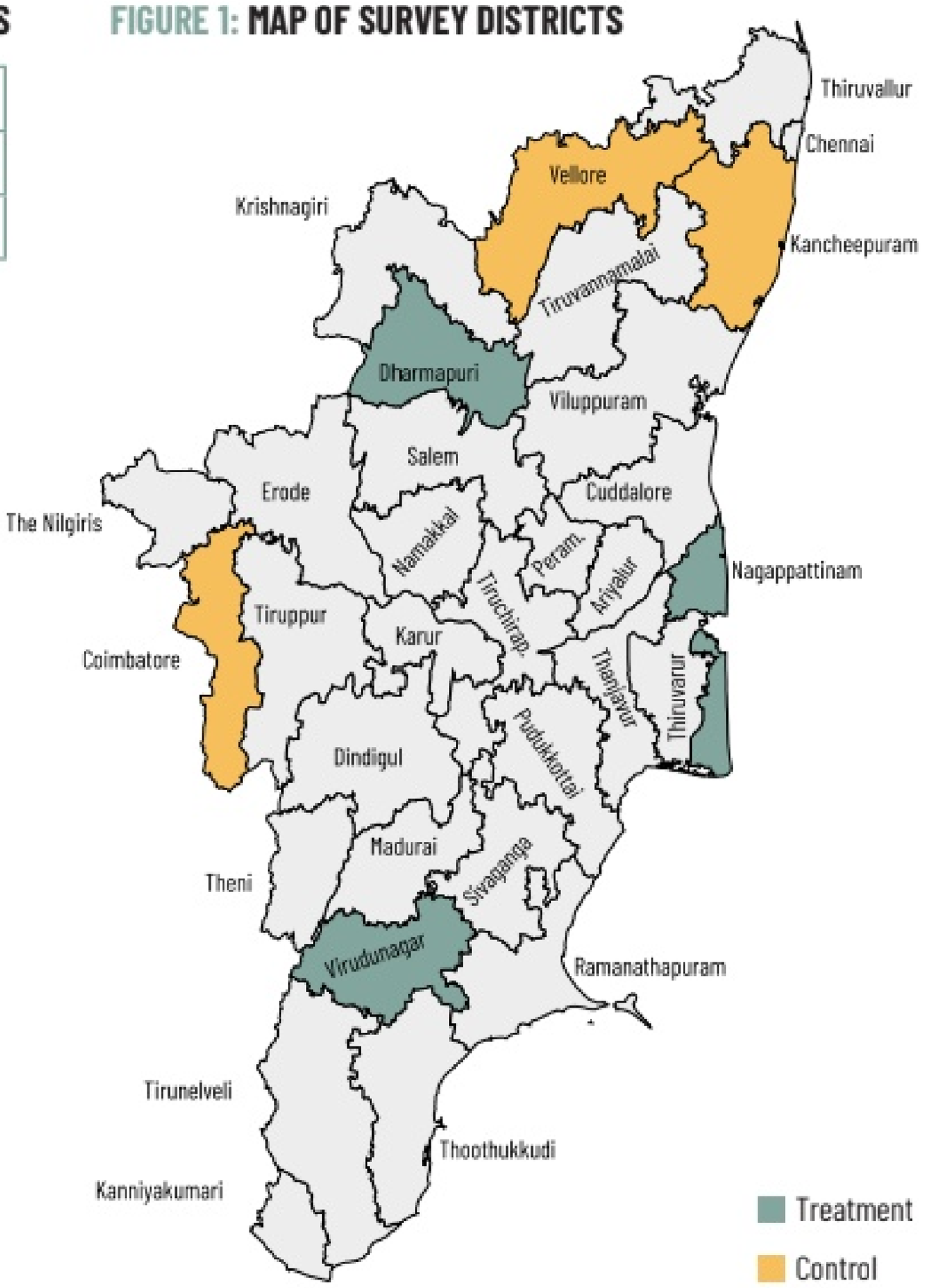
However, we encountered challenges in securing a sufficient number of beneficiaries for the study within selected villages. To address this issue, after scoping on the ground, we expanded our selection criteria to include neighbouring villages with similar demographic and economic characteristics. This approach ensured that our sample remained representative while maintaining the integrity of the treatment and control classifications we adopted.

This multi-tiered approach, beginning at the district level and refined down to the village level, ensured a robust sampling strategy that enabled meaningful comparisons between systematically chosen treatment and control groups.

TABLE 1: NUMBER OF SURVEY RESPONDENTS

Treatment Sample Size	1283
Control Sample Size	938
Total Sample	**2221**

FIGURE 1: MAP OF SURVEY DISTRICTS



SURVEY TOOL

For the quantitative survey, we compiled details on the socio-economic profile of the women, with key indicators including the respondent's age, marital status, number of family members, education, her occupation as well as that of her husband, her income as well as household income and expenditure, and land ownership. We also sought to understand the paid and unpaid work of respondents in detail.

We asked respondents about their experience with the scheme, including the process of registration, the setting up of and access to their bank accounts, their receipt and control of the transferred amount as well as their perception of the scheme's purpose. Finally, to understand the impact of the KMUT scheme, we compiled data across a variety of outcome indicators grouped into six themes:

- Expenditure patterns
- Food consumption
- Financial inclusion and wellbeing
- Women's paid and unpaid work
- Intra-household bargaining power
- Conflict and violence

POST-SURVEY FGDS

Based on an analysis of the survey data, we conducted 21 post-survey FGDs in December 2024 and January 2025 with 167 beneficiaries, across 18 villages in Dharmapuri, Virudhunagar, and Nagapattinam. The post-survey FGDs focused on clarifying specific insights from the survey data, including the beneficiaries' perception of exclusion criteria in the scheme, financial literacy, sources of income and loans, expenditure of the transferred amount, food consumption patterns, bargaining power, mobility and travel patterns, unpaid work undertaken and access to paid work, experience of domestic violence, as well as the scheme's impact on their political voice.



Photo 3: Post-Survey FGD

CHAPTER 4

SOCIOECONOMIC PROFILE OF RESPONDENTS

We compiled detailed socio-economic profiles of all respondents, including of their family. A key component of our analysis was attempting to ensure a level of socio-economic parity between the treatment and control groups. This meant that any impact observed could be attributed to the scheme itself and not because of other extraneous factors. Hence, in this section, while presenting the socio-economic profiles of the respondents, we highlight any differences between the treatment and control groups. While the treatment and control groups are similar overall in terms of socio-economic profiles, there are key distinctions, especially in terms of income. However, as discussed subsequently, we have made every effort to ensure parity while making econometric comparisons in assessing the impact of the scheme.

Both the control and treatment groups are predominantly from rural areas, with 85 % of the treatment group being rural compared to 98% of the control group and 15% of the treatment group belonged to urban areas compared to 2% of the control group.

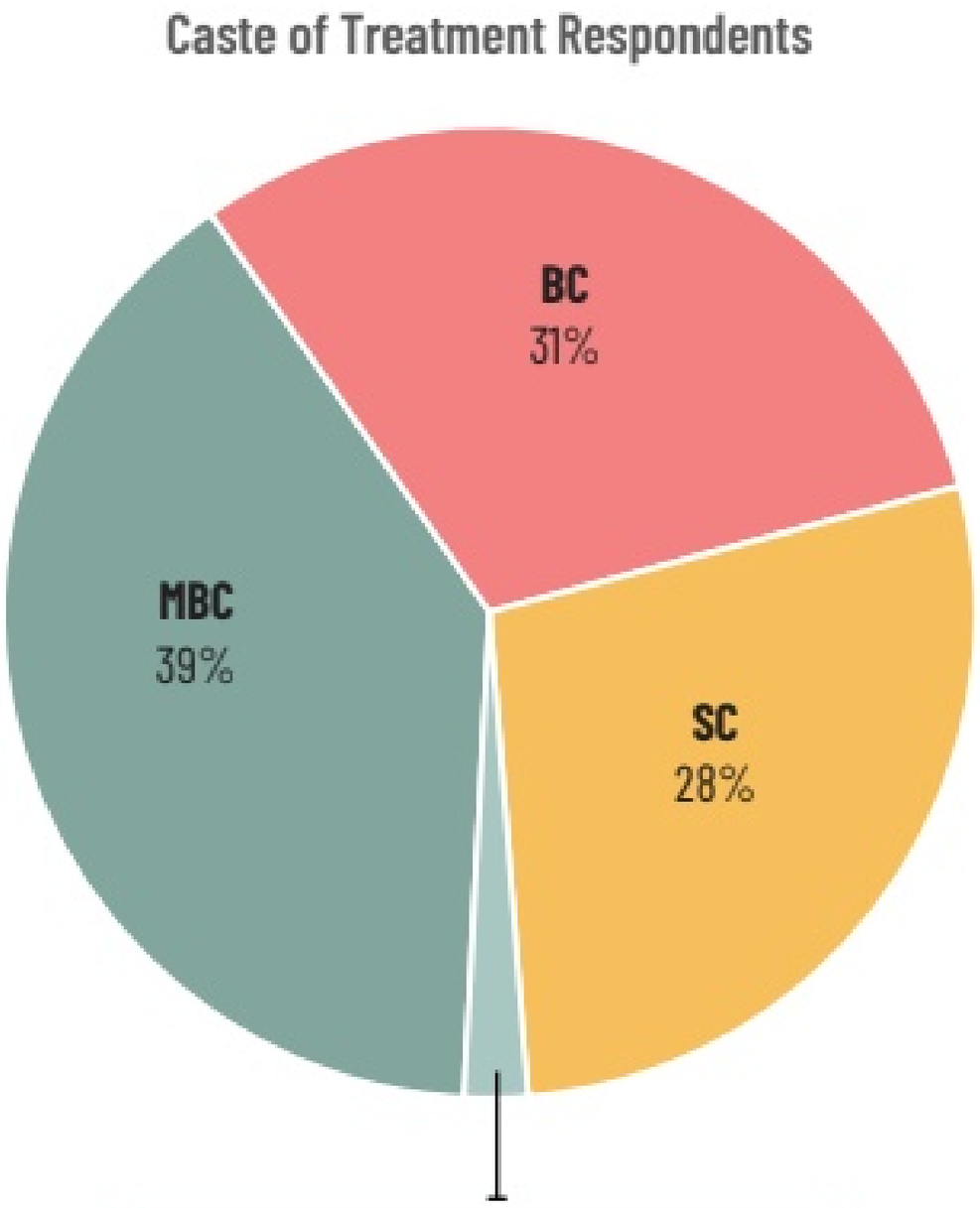
A majority of both groups were Hindu, comprising 93% of the treatment group and 92% of the control group. Treatment and control groups were also primarily from socio-economically vulnerable communities, that is, from either Scheduled Castes (SC), Backward Classes (BC), or Most Backward Classes (MBC), all of which in total comprised 98% of the treatment group and 97% of the control group (See Figure 2, p.10).

The families of the respondents were primarily nuclear, with nuclear families comprising 86% of the treatment group and 83% of the control group. 78% of the treatment group and 79% of the control group had families that comprised of four members or less.

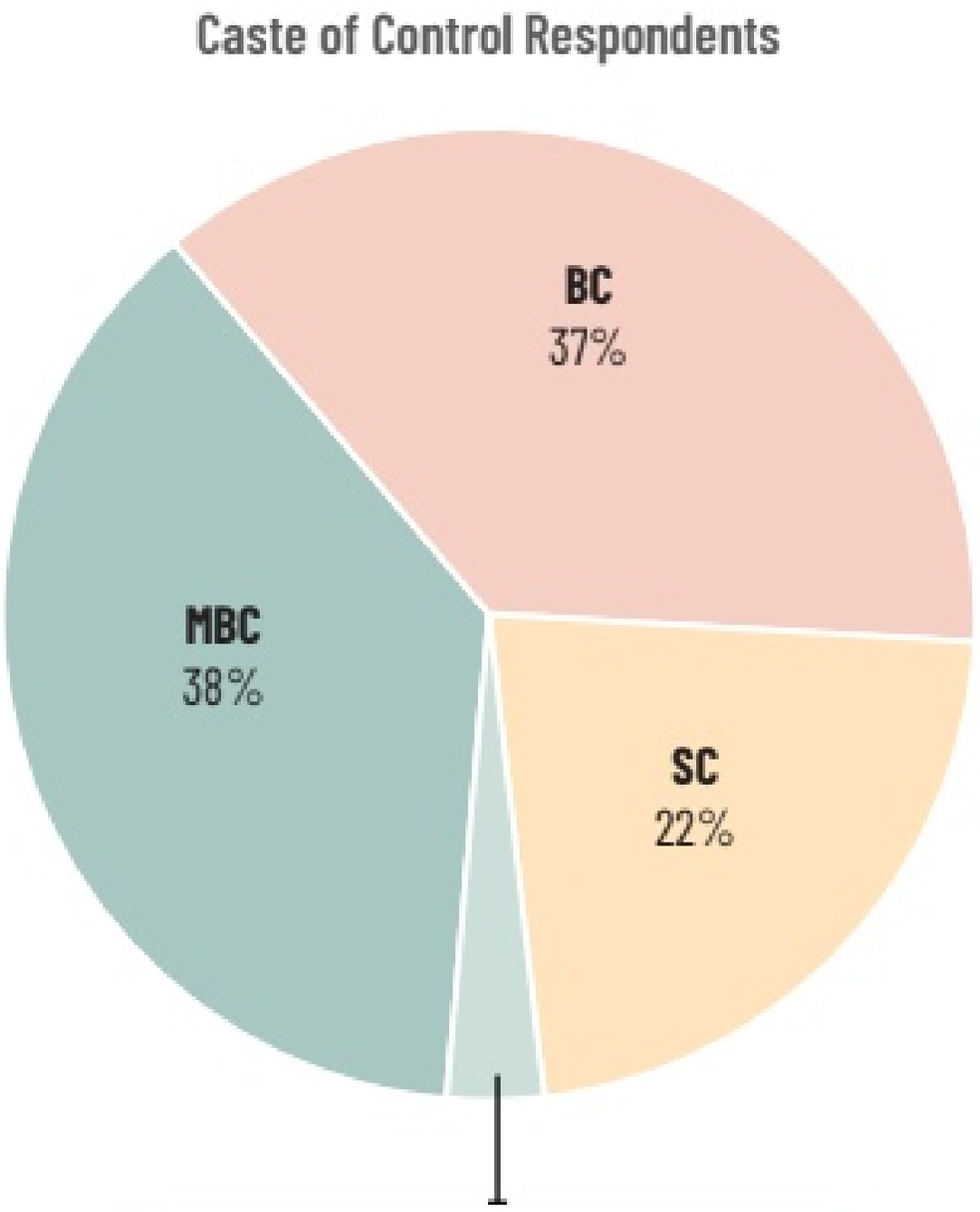
87% of the respondents from the treatment group and 85% from the control group were married at the time of the survey (including those who were remarried), with the remaining women being either widowed, divorced, separated, or deserted. 0.5% of the treatment group and 3% of the control group had never been married. 30% of the treatment group had no children, compared to 38% of the control group, with a majority of the remaining respondents having one child, more specifically, comprising 46% of the total treatment group and 47% of the control group.

51% of the treatment group stated that they did paid work outside the home compared to only 39% of the control group. (See Figure 3, p.10)

FIGURE 2: CASTE OF TREATMENT AND CONTROL RESPONDENTS

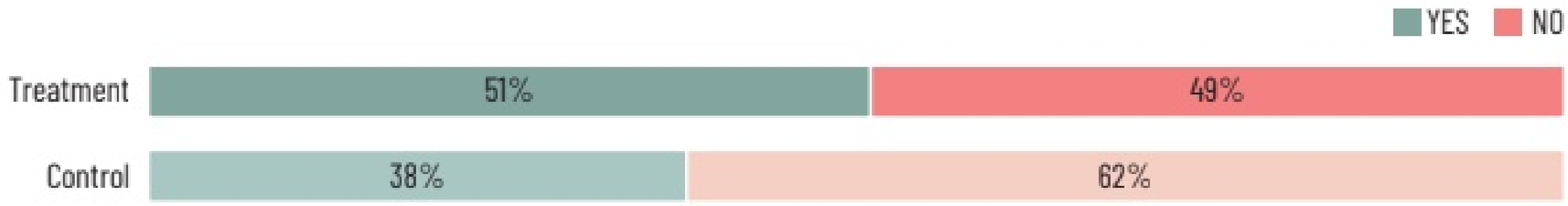


Others 2%	
ST	1.01%
General	0.55%
Other	0.31%
Unwilling to disclose / Don't know	0.23%



Others 3%	
ST	1.28%
General	1.28%
Other	0.53%
Unwilling to disclose / Don't know	0.11%

FIGURE 3: WHETHER TREATMENT AND CONTROL RESPONDENTS HAD PAID WORK



For those women who did paid work, we also inquired into the type of occupation that they were employed in across 23 categories (including a residual category labelled 'others'). Over 96% of the treatment group fell into the following six categories: non-agricultural wage labourer[9] (59%), agricultural labourer[10] (13%), scheme workers[11]

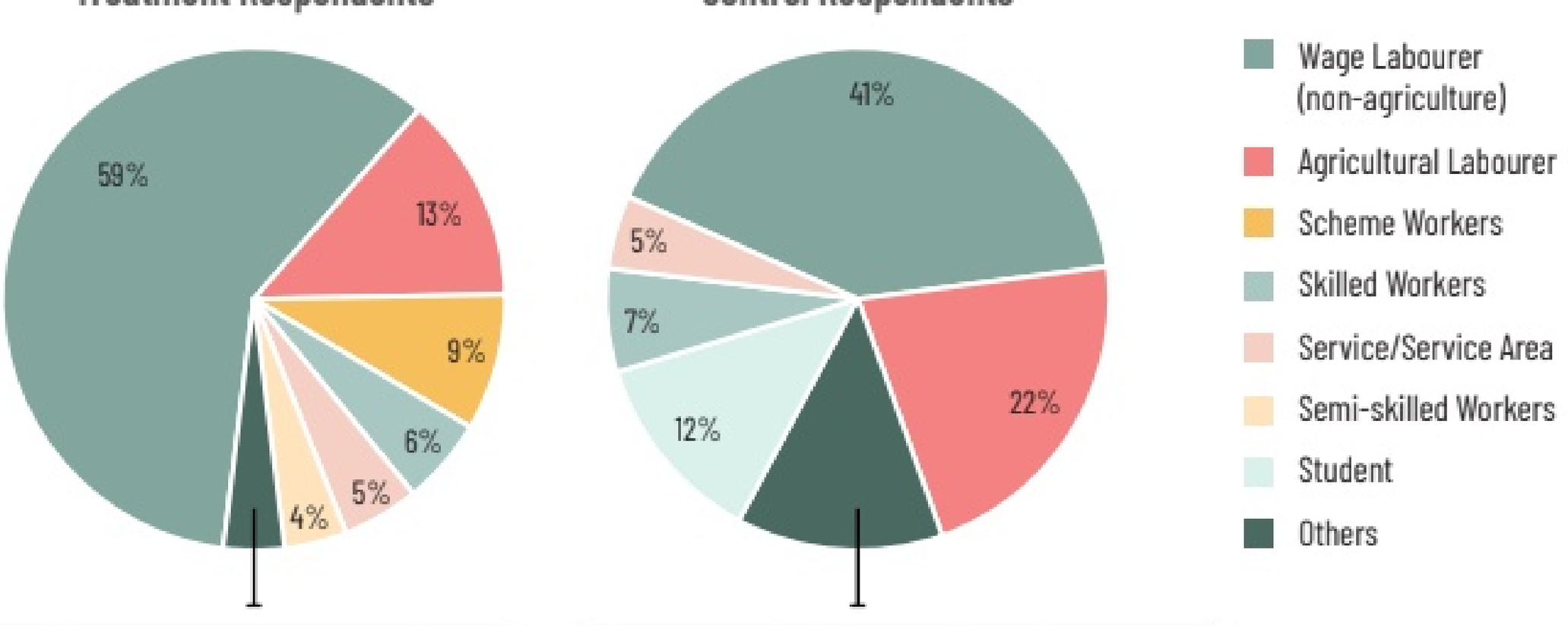
9 Including NREGA workers, rickshaw pullers, loaders, construction workers, street vendors, garbage collectors, mining, quarrying and manufacturing labourers, water carrier, sewerage.

10 Landless farm labourers

11 Including ASHAs, Anganwadi workers, Anganwadi helpers, Pashu Sakhi, Van Sakhi, Krishi Sakhi, NREGA back-office workers, midday meal scheme workers, Indira Canteen workers.

(9%), skilled workers[12] (6%), service workers[13] (5%), and semi-skilled workers[14] (4%). While a significant proportion of the control group were similarly either non-agricultural wage labourers (41%) or agricultural labourers (22%), this group also had a comparatively high proportion of students (13%). Alongside skilled workers (7%) and service work (5%), these five professions comprised 87% of the control group. Therefore, the control group had a wider range of professions that fell into the 'others' category compared to the treatment group, each comprising less than 3% of the total.

FIGURE 4: TYPE OF OCCUPATION AMONG RESPONDENTS WHO HAD PAID WORK



Treatment Group - Others 4%	
Small/Temporary Business	1%
Lower Professionals	0.9%
Other	0.6%
Small Trader	0.5%
Farmer	0.3%
Homemaker but doing part time job	0.2%
Government Class IV	0.2%
Big and Medium Traders	0.2%

Control Group - Others 13%	
Other	3%
Lower Professionals	3%
Small/Temporary Business	2%
Farmer	2%
Homemaker but doing part time job	1%
Semi-skilled Workers	0.6%
Government Class IV	0.6%
Big and Medium Traders	0.6%
Animal Husbandry Work/Forestry	0.6%
Government Managerial Job	0.3%
Government Administrative Job	0.3%

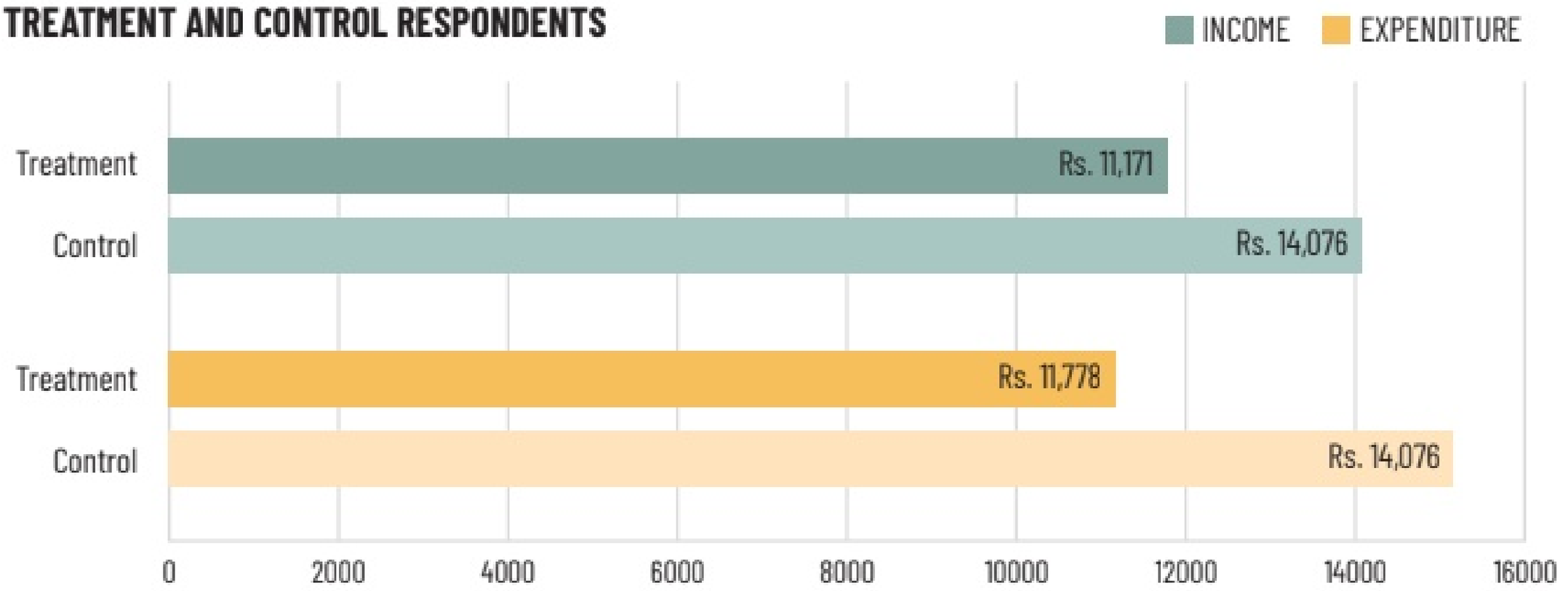
12 Including driver, mechanic, electrician, gas fitter, steam supply, air conditioning supply, meter reading, waste management, water supply, mobile technician, appliance repairer, plumber, tailor, cobbler, carpenter, sailor, gardeners, fishery workers, miners, stone cutter and carvers, sculptors, painters, welders, sheet metal workers, blacksmith, goldsmith, glass makers, handicraft workers, printing and related workers, garment factory worker, dyeing.

13 Including cook, waiter, washerman, barber, domestic worker, childcare worker, elder care worker, carer for differently abled, beautician, hairdresser, chowkidar, travel attendants/guides, shop salespersons and demonstrators, delivery boy, private guard, safai karamchari, personal assistant, banking correspondent, insurance agent, CSP agent.

14 Including artisans, bricklayers, potters, stone cutters, furniture, basketry, mat makers, food processing workers.

The mean income and expenditure of the respondents' households are also indicative of their economic status, with the data showing that the control group had a higher income and expenditure compared to the treatment group.

FIGURE 5: MEAN INCOME AND EXPENDITURE OF TREATMENT AND CONTROL RESPONDENTS



A majority of the households held ration cards, with 89% of the treatment group and 78% of the control group holding Below Poverty Line (BPL) cards; and 11% of the treatment and 6% of the control group holding *Antyodaya Anna Yojana Cards* (Tamil Nadu Civil Supplies Corporation, 2020). It's worth noting that 14% of the control group did not know their ration card status, which could explain the differences between the two groups.

60% of the treatment group and 56 % of the control group lived in *pucca* houses, while 31% of the treatment group and 39% of the control group lived in *pucca-kuccha* houses. The remaining 9% of the treatment group and 5% of the control group lived in either *kuccha* houses or huts. 78% of the treatment group's family and 85% of the control group's family owned a house, but only 12% of the treatment and 21% of the control group's families owned any agricultural land.

20% of the treatment group and 12% of the control group stated that they were illiterate or had never attended school. 11% of the treatment group and 8% of the control group had attended primary school, while 46% of the treatment group and 46% of the control group had attended secondary school. The remaining 23% of the treatment group and 34% of the control group had either attended higher secondary school or further studies, including a small proportion of postgraduates.

CHAPTER 5

ACCESS TO THE SCHEME

We specifically asked the 1283 beneficiaries detailed questions about their experience in accessing the KMUT scheme and the transferred amount. We sought to determine if they could easily register for the scheme as well as whether they had access to and control of the money received – key aspects in understanding the effectiveness of the scheme in facilitating their financial independence. We found that the scheme was effectively introduced and implemented, with a vast majority of the surveyed beneficiaries finding the process of registration easy and regularly receiving the full amount they are owed.

REGISTRATION FOR KMUT

We asked the treatment group on a Likert scale how they found the process of registering for the KMUT scheme (see Koo and Yang, 2025). 90% of the respondents said they found the process either very easy or easy, but 9% stated they had difficulties in registering for the scheme. It's worth noting that 81% of the respondents said that they had help with the registration process, with only 19% completing the process on their own.

FIGURE 6: BENEFICIARIES' PERCEPTION OF KMUT REGISTRATION PROCESS

Q. How did you find the process of registering for the Urimai Thogai Scheme?

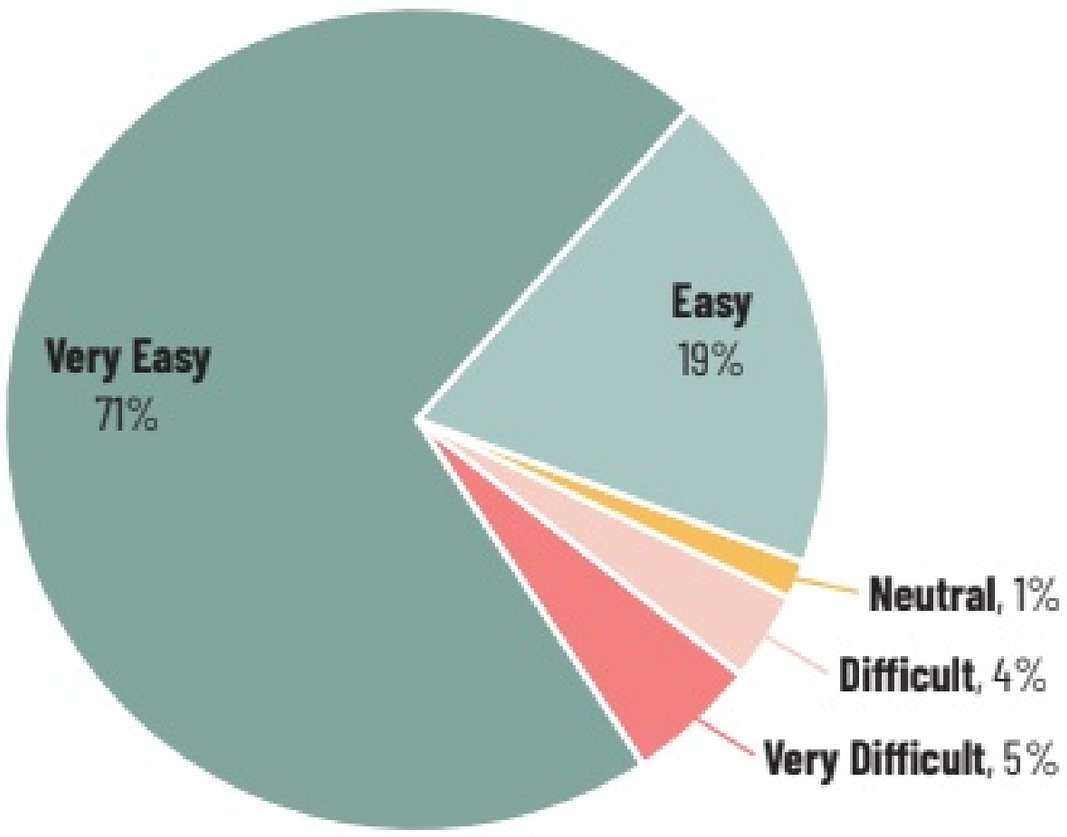
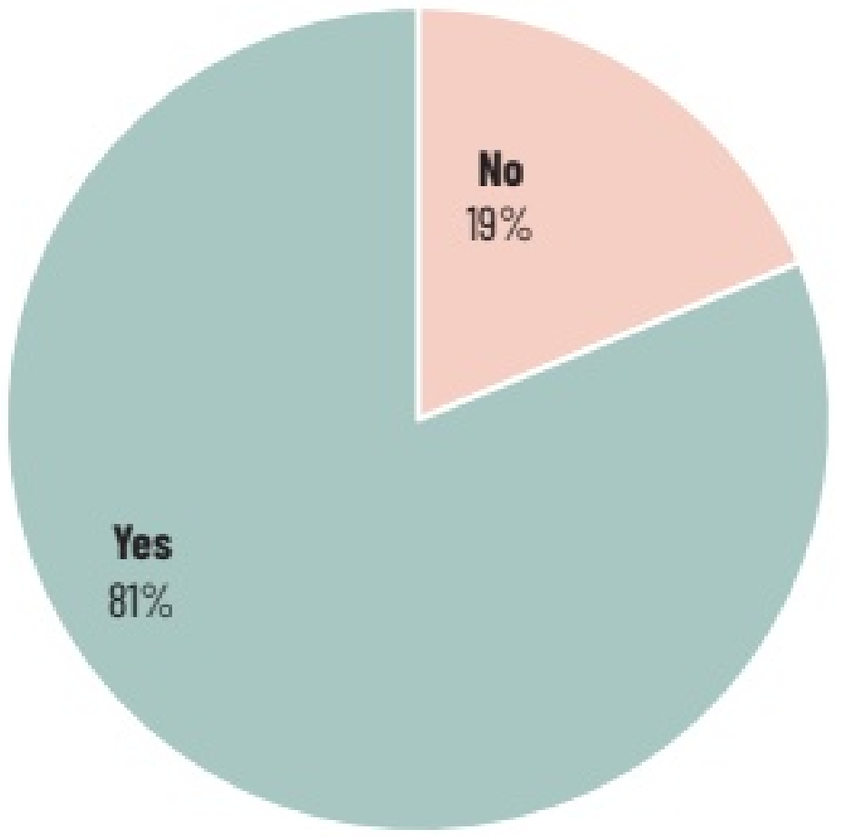


FIGURE 7: HELP FOR BENEFICIARIES DURING KMUT REGISTRATION PROCESS

Q. Did anyone help you with the registration?



FINANCIAL INCLUSION

In terms of financial inclusion, 96% of the respondent beneficiaries had a bank account prior to enrolling in the scheme. 63% of the treatment group had an ATM card, out of which 77% said they held their own ATM card, while 37% did not have an ATM card at all. 67% of the female beneficiaries owned a mobile phone, out of which 50% owned a smartphone. 61% of the treatment group stated they were able to read SMS or text messages, with 39% unable to do so.

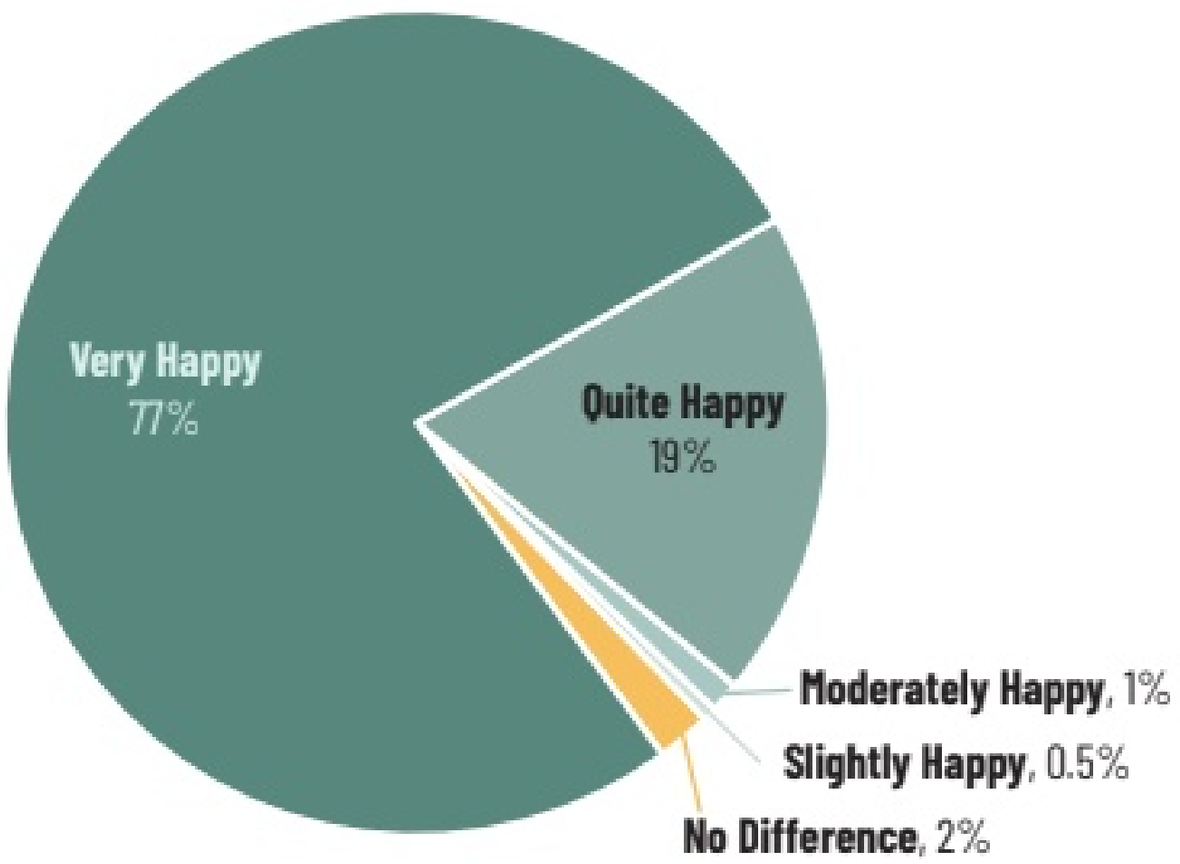
PERIODICITY OF PAYMENT

99% of the women stated that they received the KMUT amount on the 15th or 16th of the month and 99% stated they got the full amount of Rs. 1000[15] each month. Among the 1% respondents who stated they did not receive the full amount, a majority (65%) believed that it was because of banking charges or for not maintaining the minimum balance, while 35% did not know why this error took place. Overall, the fact that a majority of surveyed beneficiaries received the full amount regularly indicates the efficient and timely implementation of the scheme.

91% of the respondents stated that they received an SMS on receipt of the KMUT amount, including some women who did not own a phone but would still get the SMS (such as on a family member's phone). We asked them, on a Likert scale, how they felt when they received this SMS: 98% said they felt very happy or happy on receiving the SMS. Furthermore, 90% of the treatment group planned expenditure in anticipation of receiving the KMUT amount. While we discuss the specific impacts of the scheme in further detail below, it is important to recognise the value that receiving the KMUT amount holds for these women, including that they do (and are able to) plan consumption around the receipt of the amount.

FIGURE 8: BENEFICIARIES' FEELING ON GETTING SMS CONFIRMING RECEIPT OF KMUT AMOUNT

Q. How do you feel when you get the SMS?



CONTROL OVER KMUT MONIES

99% of the treatment group said they operate their own bank account, of which 56% said that they did so with the help of bank staff and family members. 99% of the treatment group said that they have complete control over the KMUT amount. Of the 1% who did not have complete control, in 75% of such cases their husband controlled the transferred amount, with the father or children controlling the money of the remaining women. The fact that nearly all women stated they had complete control over the KMUT amount is indicative of the success of the scheme in facilitating a measure of financial independence for the beneficiaries.

15 Approximately 12 USD.

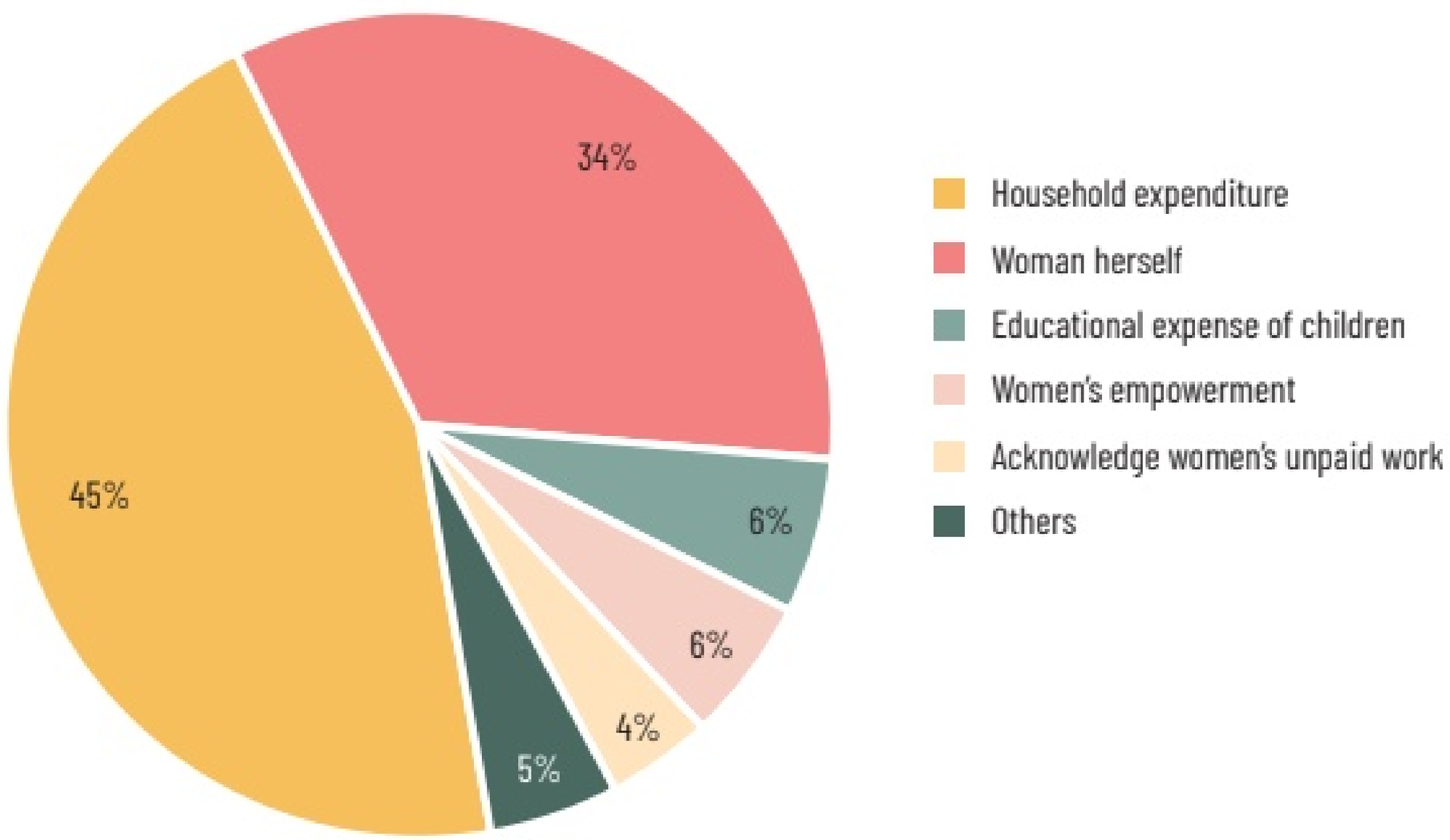
CHAPTER 6

PERCEPTION OF THE SCHEME

The government notification for the KMUT scheme is rare among cash transfer schemes for explicitly stating that one of its key objectives is to recognise women's UDCW. An important component of our study was to determine if this messaging was reaching the beneficiaries of the scheme. In the survey, we therefore asked the women the purpose of the cash transfer.

About half the women believed that the transfer was either for household expenditure (46%) or for their children's educational expenses (6%); while a third of the women (34%) believed that it was intended for them. Only about 10% of the respondents understood the transfer as either being for women's empowerment (6%) or as recognition of their UDCW (4%). 5% stated the money was given for other reasons, with respondents either not knowing the reason or believing it was for political reasons, for women's needs generally, to help poor people, or to help the population more broadly.

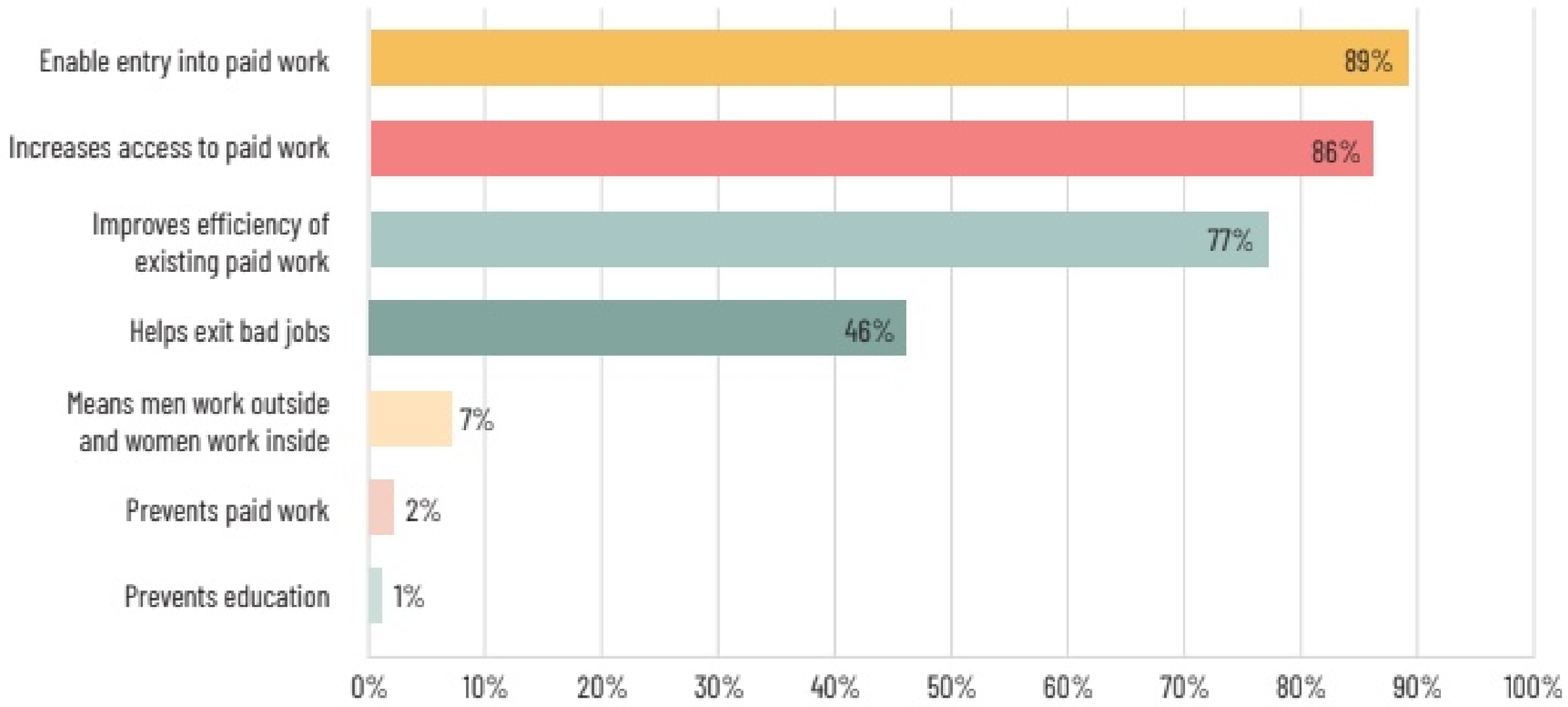
FIGURE 9: BENEFICIARIES' PERCEPTION OF KMUT'S PURPOSE



The electoral promises of UCTs led to debates among feminists (Chandrasekharan, 2021; Geetha, 2021; Langer, 2021; Mustafa, 2021; Neetha, 2021; Reddy, 2021; Tulsyan, 2021; TNN, 2022). Some feminists argued that the schemes would result in greater demands for unpaid work and reinforce gender stereotypes (Chandrasekharan, 2021; Khaitan, 2021). They also believed that the UCTs would detract from the better measurement of women's unpaid work and from much needed investments in physical infrastructure such as piped water, clean energy, and transportation as well as care infrastructure in the form of universal, high-quality child-care that could reduce UDCW (UN Women, 2019; Himanshu, 2024; 2025; Mitra, Puri, and Ramesh, 2024; Venkatesan, 2025; Chandhoke, 2025). Another concern was that the transfers would disincentivize women from seeking paid employment and getting an education (Khaitan, 2021). Many of these feminists argued that the focus of welfare programmes instead ought to be on improving access to paid employment and redistributing UDCW between men and women (Khaitan, 2021; Neetha, 2021; Tulsyan, 2021; TNN, 2022).

In light of these concerns, we asked the female beneficiaries themselves what they perceived the impact of the KMUT scheme to be. An overwhelming majority associated KMUT with positive effects, perceiving that it either enabled entry into paid work (89%), increased access to paid work (86%), or improved the efficiency of existing paid work (77%); with almost half feeling that it had helped them exit bad jobs (46%). We found that a very small minority said that KMUT had negative effects: only 7% felt that KMUT reinforced the gendered division of labour (that men work outside and women work in the home) and only 1-2% of our respondents felt that KMUT either prevented paid work or education.

FIGURE 10: BENEFICIARIES' PERCEPTION OF KMUT'S IMPACT



CHAPTER 7

IMPACT OF KMUT

To evaluate the impact of the KMUT scheme, we compared the following key outcomes between the treatment group of KMUT beneficiaries and the control group of respondents who did not receive the scheme benefits:

1. Expenditure patterns
2. Food consumption
3. Financial inclusion and wellbeing
4. Women's paid and unpaid work
5. Intra-household bargaining power
6. Conflict and violence

METHODOLOGY FOR DATA ANALYSIS

In our analysis of the scheme's impact, we began with propensity score matching, which is a well-established methodology to mimic the effects of randomisation in circumstances when a randomised control trial is either not feasible or not ethical, as was in this case (Dehejia and Wahba, 2002; Austin, 2011). This method involves the creation of an artificial counterfactual by matching treated units with non-treated units of similar characteristics. The impact of the intervention is calculated by estimating the average difference in outcomes between the matched treatment and control units.

In our study, we paired individuals from the treatment group with similar individuals from the control group based on relevant socio-economic characteristics such as age, marital status, and family size. We had a sufficiently large dataset and detailed socio-economic profiling of respondents, which could ensure the validity of this method of analysis. Matching the treatment and control group based on these characteristics helped reduce the effect of any differences in characteristics on the outcomes, which means the only difference between the two groups that remained is whether they received the KMUT transfer amount or not. This means that any differences in outcomes, as documented below, are likely due to the scheme and not any other factor.

We followed this analysis with the more robust method of entropy balancing, which goes beyond simply matching the two groups, but further assigns weights to units within the group and balances the distribution of each group (Hainmueller, 2012; Källberg and Waernbaum, 2023). This means that this method balances out the average as well as the outliers in each group, so that the dataset as a whole is made equivalent and comparable – ultimately ensuring the credibility of any causal inferences that are made. We consider results at the 95th percentile as statistically significant.

EXPENDITURE PATTERNS

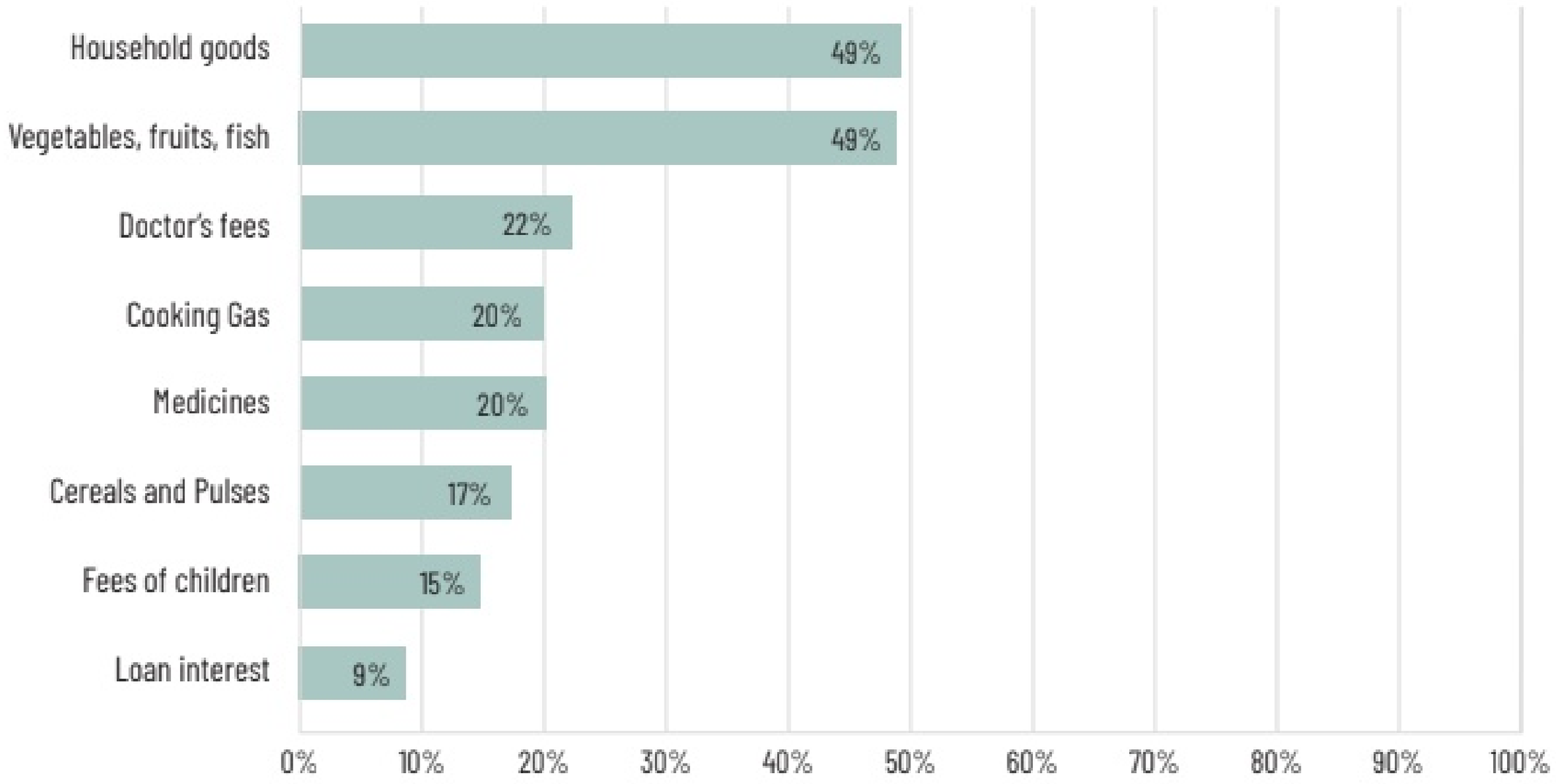
During the survey, we collected detailed data on how beneficiaries and control group respondents utilised their financial resources. We found that beneficiaries primarily spent the KMUT amount they received, with 82% of the treatment group stating they spent the money, 6% saving the full amount, and 12% partially saving and partially spending the amount. We asked the beneficiaries what they spent the money on across 25 different categories of expenses (including 'others'). About half the women said they spent the money on household goods or food items, with a fifth of the women using it for medicines, doctors' fees, or cooking gas.

> *"I use the money to pay for the groceries; I would buy them in advance and pay back the shop owner once I get this 1000 rupees."*

> *"We get vegetables, spend for the household, get rice. We use it when our regular income or hands on cash is low."*

FIGURE 11: **UTILISATION OF KMUT AMOUNT**

Q. What do you use the money for within the household?



What do you use the money for within the household? (Others)					
Expense	Yes	Expense	Yes	Expense	Yes
Others	4%	Spent on self	3%	Education/skill development	0.6%
Travel fare	4%	Rent	2%	Water	0.4%
SHG membership fee	3%	Mobile phone Recharge	2%	Maintain livestock	0.3%
Electricity	3%	SHG Loan repayment	2%	Jewellery	0.1%
On festivals	3%	For vehicle	0.6%	Farming and Purchasing Livestock	0%

We then compared the expenditure patterns between the treatment and control group, to determine how receipt of the KMUT amount might have shifted spending and consumption patterns for beneficiaries.

HOUSEHOLD SPENDING

Based on our comparison of the treatment and control groups, we found that the scheme led to an increase in spending on medicines, but with reduced spending on food, household goods, and personal needs. The beneficiaries are also less likely to spend on loans and loan repayment, which may be indicative of the support the KMUT scheme provides financially.

> *"I use it for the cylinders. Even now I had given the amount for the cylinder. This amount is useful for the cylinders and for me to get tablets for my son."*

> *"I am pregnant and I use the money for scanning and support for household expenses, cylinder booking etc."*

More specifically, the scheme meant that the beneficiaries are:

FIGURE 12: **IMPACT OF KMUT AMOUNT ON HOUSEHOLD SPENDING**

20 percentage points ↑ more likely to spend on medicines

17 percentage points ↓ less likely to spend on loans

9 percentage points ↓ less likely to spend on SHG loan repayment

35 percentage points ↓ less likely to spend on vegetables, fruit, and fish

35 percentage points ↓ less likely to spend on cereals

29 percentage points ↓ less likely to spend on doctor fees

28 percentage points ↓ less likely to spend on household goods

19 percentage points ↓ less likely to spend on personal needs

SELF-EXPENDITURE

Furthermore, the data showed reduced spending by beneficiaries on self-expenditure for clothes, beauty products, entertainment, and food compared to the control group. It reveals that for the beneficiaries, the KMUT amount primarily benefits the household and the family, rather than leading to any expenditure for their own needs.

> *"Even if I get flowers for myself, it is 40 rupees. Imagine how much it would cost if I adorned myself with flowers every day. 100 jasmine flowers are 80 rupees at today's rate."*

In particular, we found that:

FIGURE 13: IMPACT OF KMUT AMOUNT ON EXPENDITURE FOR SELF

Clothing expenditure ↓ decreases by **14 percentage points**

Health-related spending ↓ decreases by **10 percentage points**

Food expenditure ↓ decreases by **7 percentage points**

Spending on cosmetics and beauty products ↓ decreases by **5 and 0.9 percentage points respectively**



Photo 4: Survey Interview with Respondent

In focus group discussions (FGDs), the pain of inflation and price rises on women's lives became abundantly clear. As one of the women explained during the FGDs:

> *"We don't take the money for ourselves, we only think we have to pay for the cylinder or take care of the kids and so on. The reason is that the prices of the essential commodities are skyrocketing. We are not able to manage the expenses. Even the cylinder is an essential commodity, and we must keep some money aside for it. I must finish cooking by 6 or 630 and I have to spend money on the tablets. I need to get up at 4 am to even manage this schedule, to milk the cows by 7 am. Hence, I spend the majority of the KMUT money on the cylinder. I am not able to spend money for myself because of the rates of things. We are 6 of us and one litre of oil is 150 rupees. How do you think I can manage these expenses? How can I take it for myself? In case I have money remaining from the 100 days job then I can spend something for myself."*

While the provision of the KMUT amount may not have led to dramatic shifts in spending patterns, it appears to provide an integral buffer against the effects of inflation.

> *"I would use it (cylinder) for 10 nights and 10 days, if I cook only regular food it would last. If I consciously restrict it to 2 meals, it comes for 30 days, that's all."*

Against these inflationary pressures, women also expressed altruistic preferences. As they shared,

> *"We prioritize the children. We sometimes want to spend for ourselves, but we would think, 'are my needs bigger than my child's need' and then we defer."*

> *"We spend it for books and so on, we have not kept aside 5 rupees for ourselves. I spend for the medicines and it gets exhausted."*

FOOD CONSUMPTION

When it came to food consumption specifically, despite the modest amount paid under the scheme, there were notable shifts in patterns of food consumption when comparing the treatment and control group. We found a mixed effect across the 9 food groups: with increased frequency of consuming fish, fried foods, and aerated drinks among the beneficiaries and their household members; and reduced frequency for other food groups including pulses, dairy products, vegetables, and fruit. We also found reduced frequency of meals (breakfast, midday meals and dinner) taken by beneficiaries. It is surprising but the frequency of consumption of some food groups as well as the consumption of many specific food groups and meals declined, which does seem anomalous.

FOOD CONSUMPTION - KMUT BENEFICIARIES' HOUSEHOLD MEMBERS

Overall, beneficiaries' household members consumed more fish and eggs; but consumed less sweets, lentils, and other vegetables. While the reduced consumption of sweets is a potentially positive indicator, it is of concern that healthy food groups such as vegetables and lentils have seen a decrease in consumption. However, the increased consumption of fish and eggs could indicate that the KMUT scheme helps women purchase a greater variety of food.

In particular, beneficiaries' household members are:

FIGURE 14: **IMPACT OF KMUT AMOUNT ON HOUSEHOLD FOOD CONSUMPTION**

19 percentage points ↑ more likely to consume fish

8 percentage points ↑ more likely to consume eggs

4 percentage points ↑ more likely to consume meat

3 percentage points ↑ more likely to consume root vegetables

2 percentage points ↑ more likely to consume green vegetables

19 percentage points ↓ less likely to consume sweet food

12 percentage points ↓ less likely to consume vegetables (other than root and green vegetables)

10 percentage points ↓ less likely to consume lentils

7 percentage points ↓ less likely to consume milk

4 percentage points ↓ less likely to consume fruit

We also specifically examined the frequency of consumption for different kinds of foods. We found that the beneficiaries' household members consumed fish, fried food, and aerated drinks more frequently, but reduced the frequency of consuming dairy, pulses, vegetables, and fruit. In many ways, this reflects the patterns observed in food consumption generally, with a potentially greater variety of food groups being consumed but a concerning reduction in the frequency of healthy food groups.

> *"This amount is insufficient! One kg of mutton is 800 rupees. Will this 1000 rupees be sufficient then. We must take a loan of 500 rupees to even buy meat!"*

In particular, beneficiaries' household members exhibit the following tendencies:

FIGURE 15: IMPACT OF KMUT AMOUNT ON HOUSEHOLD FOOD CONSUMPTION FREQUENCY

Frequency of consuming fish ↑ increases by **0.4 times per week**

Frequency of consuming dairy products ↓ decreases by **0.3 times per week**

Frequency of consuming pulses ↓ decreases by **0.4 times per week**

Frequency of consuming green vegetables ↓ decreases by **0.3 times per week**

Frequency of consuming fried foods ↑ increases by **0.2 times per week**

Frequency of consuming fruit ↓ decreases by **0.2 times per week**

Frequency of consuming aerated drinks ↑ increases by **0.2 times per week**

FOOD CONSUMPTION - KMUT BENEFICIARIES

The beneficiaries themselves consume fewer meals, again indicating that the KMUT amount may not go to their own expenses and needs. They are:

FIGURE 16: IMPACT OF KMUT AMOUNT ON BENEFICIARIES' MEAL CONSUMPTION

8 percentage points ↓ less likely to have breakfast

5 percentage points ↓ less likely to consume a midday meal

4 percentage points ↓ less likely to to have dinner

The rising cost of groceries has resulted in the reduced frequency of meals. As one woman explained in an FGD:

> *"We need so many things to even cook proper food. It is difficult to even afford all the groceries to cook food. That is why it's difficult to even cook two full meals, and that's why we restrict it to two…We can eat properly only when we are able to procure the groceries beforehand. It must be readily available at the time of cooking. So, eating twice is not our preference, but because we need to cut our expenses. Yes, we are eating twice only to cut costs. If we had money, we would have eaten 6 meals like asked. We could even eat throughout the day. When we don't have the necessary groceries, we can only eat twice."*

In one FGD, the stress of paying back loans was so acute that women said they were unable to eat.

> *"Even if we take a portion of that money to make a curry, we must double think the decision. Our future hassle of dealing with the money lenders immediately flashes in front of our eyes."*

In terms of the type of food consumed, beneficiaries follow the same pattern as their households and are more likely to consume fish but have a reduced intake of vegetables, pulses, and sweet food. In particular, beneficiaries are:

FIGURE 17: IMPACT OF KMUT AMOUNT ON BENEFICIARIES' FOOD CONSUMPTION

17 percentage points ↑ more likely to consume fish

23 percentage points ↓ less likely to consume sweet food

13 percentage points ↓ less likely to eat lentils

12 percentage points ↓ less likely to have vegetables

In terms of the frequency of foods being consumed, the beneficiaries again exhibited similar patterns to that of their household members by consuming fish, fried food, and aerated drinks more frequently; but showing reduced intake of pulses and vegetables. In particular, for beneficiaries:

FIGURE 18: IMPACT OF KMUT AMOUNT ON BENEFICIARIES' FOOD CONSUMPTION FREQUENCY

Frequency of consuming fish ↑ increases by **0.4 times per week**

Frequency of consuming pulses ↓ decreases by **0.3 times per week**

Frequency of consuming green vegetables ↓ decreases by **0.3 times per week**

Frequency of consuming fried foods ↑ increases by **0.2 times per week**

Frequency of consuming fruit ↓ decreases by **0.2 times per week**

Frequency of consuming aerated drinks ↑ increases by **0.2 times per week**

When we probed women's food preferences in the FGDs, especially whether and why KMUT beneficiaries were not using the cash transfer to eat healthier food, they claimed that the amount was insufficient saying that:

> *"We are only able to buy vegetables, tomatoes, oil etc. We get lentils from the ration shop. We cannot buy meat or eggs. We might just buy it rarely or for some festival."*

> *"If a family wants to eat good food, this 1000 rupees can be sufficient for only one day."*



Photo 5: Survey Interview with Respondent

FINANCIAL INCLUSION

> *Earlier only the men were considered to be the head of the household, now the women can be too. The moment the woman's photo is on the ration card it feels that we have a lot of respect and responsibility. Even the men are asking the photos to be changed to their wives so as to get this 1000 rupees."*

FINANCIAL LITERACY AND ACCESS TO FINANCIAL INFRASTRUCTURE

The twin objectives of the KMUT scheme are to recognise work that women perform for their families and to improve their livelihood, standard of living, and prospects for a life with self-respect (Government of Tamil Nadu, 2023a). Financial independence is key to these aspirations. Hence, we examined how KMUT impacts financial inclusion, financial well-being, and access to financial infrastructure. Beneficiaries showed increased usage

of banks and greater financial awareness as well as greater control over finances. They withdrew their KMUT monies from banks, ATMs, customer service points, the post office, and the e-Sevai centres. However, they did not participate in noncustodial banking activities including accessing savings, investing, or processing loans. More specifically, beneficiaries are:

FIGURE 19: IMPACT OF KMUT AMOUNT ON FINANCIAL LITERACY AND ACCESS TO FINANCIAL INFRASTRUCTURE

19 percentage points ↑ more likely to be able read the passbook

18 percentage points ↑ more likely to withdraw money from ATMs

15 percentage points ↑ more likely to be aware about banks

13 percentage points ↑ more likely to to visit a bank

12 percentage points ↑ more likely to have access to banking services

3 percentage points ↓ less likely to process loans, but this difference is not statistically significant

9 percentage points ↓ less likely to access savings, but this difference is not statistically significant

6 percentage points ↓ less likely to make investments



Photo 6: KMUT ATM Card

However, when it came to their treatment at the bank, beneficiaries noted that

> *"The bank only takes care of those who bring money to deposit. People who invest in lakhs are only respected. They will never treat us with respect. They will always ask 'why are you in such a hurry'."*

Similarly, when it came to formal loans, they pointed out that

> *"We don't have collateral. Only people with collateral can access these loans. We just have a hand to mouth existence, no properties so we can't get it."*

However, when we queried if they would be interested in becoming more financially literate, many women also observed that the current system worked for them. As one woman explained during the FGDs:

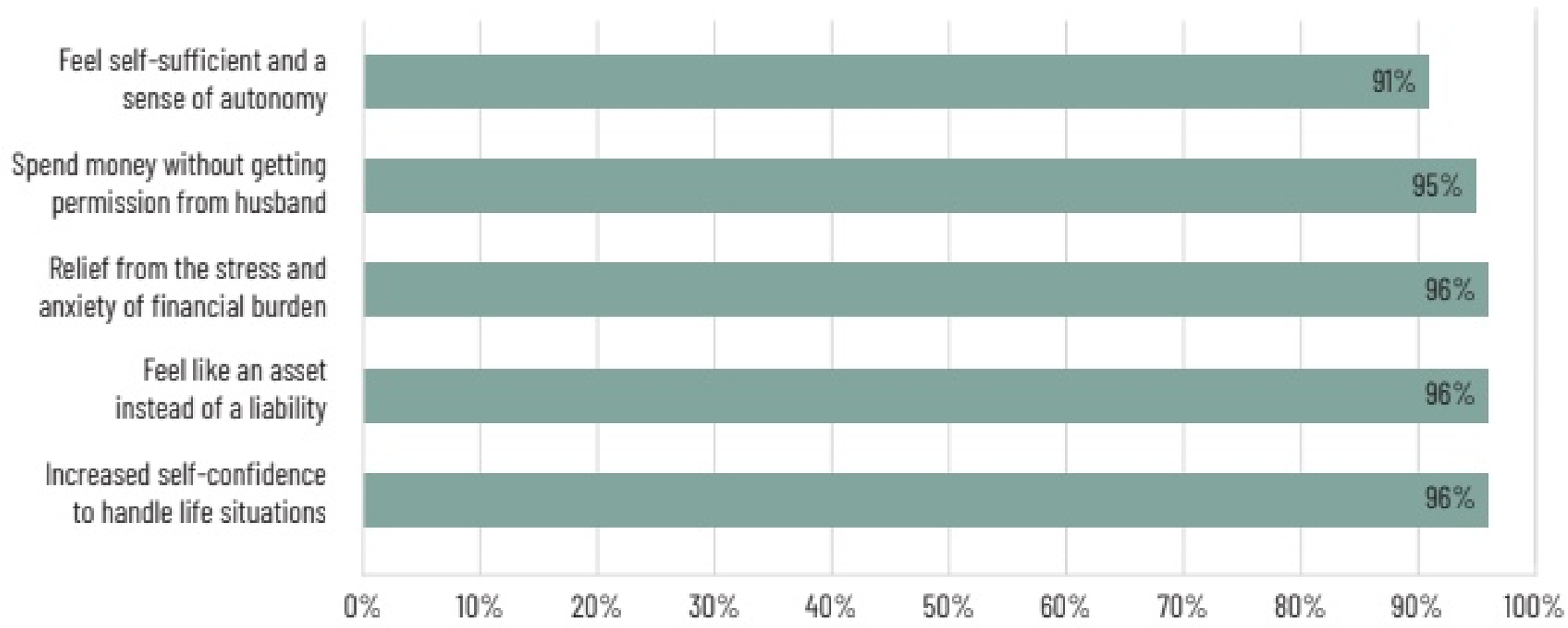
> *"Yes, we understand but after the 100 days' work has come, and Aadhar has been linked, it is easy to use thumb impression to access the payment. They help us take the salary at our work site or even come to our homes for the transaction."*

FINANCIAL WELLBEING

In our pre-survey FGDs, women spoke about being at peace with the KMUT money in hand. This led us to formulate questions on financial well-being. We identified a significant positive impact on all measures of financial well-being for beneficiaries as a result of KMUT.

We found the following results:

FIGURE 20: BENEFICIARIES' PERCEPTION OF KMUT'S IMPACT ON FINANCIAL WELLBEING



More specifically, beneficiaries when compared with control group respondents shared the following insights.

> *Earlier we used to expect our husbands to buy us stuff, nowadays after this 1000 rupees we are able to buy things or food for ourselves whether the husbands buy it or not. Sometimes we buy fruit from the pushcart for ourselves and our children."*

> *I buy Boost for the kids, and I love having it. I spend 300 rupees for it. Earlier we will not buy such items if we don't have cash. Now I get it regularly after this KMUT."*

FIGURE 21: IMPACT OF KMUT AMOUNT ON FINANCIAL WELL-BEING

Ability to handle financial stress and burdens ↑ increases by **25 percentage points**

Ability to handle overall life situation ↑ increases by **20 percentage points**

Feeling of being an asset to the family ↑ increases by **20 percentage points**

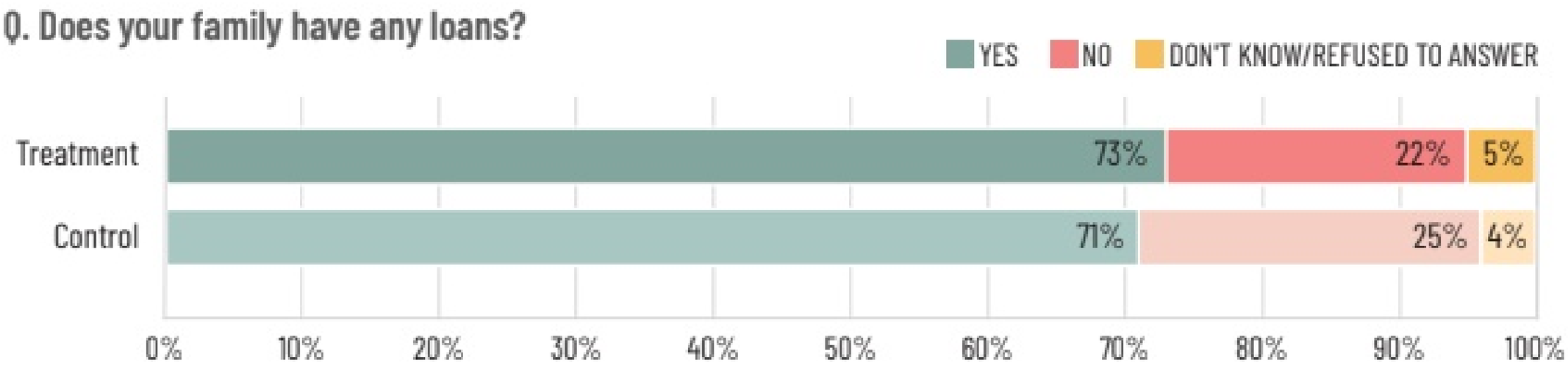
Financial self-sufficiency ↑ increases by **19 percentage points**

LOANS

> *We will manage with loans till this amount comes, once the amount comes, we will pay back the loan."*

We found that 73% of the beneficiaries and 71% of the control group respondents had loans. On average the control group had loans with an average value of 2.64 lakhs[16] while the treatment group had loans with an average value of 2.09 lakhs[17].

FIGURE 22: WHETHER TREATMENT AND CONTROL RESPONDENTS HAVE LOANS



> *They know that the women will at least work as a coolie and pay back the loan. See for this scheme also they wanted the women to change the cards to their name.*

16 Approximately 3,000 USD.

17 Approximately 2,500 USD.

Respondents in the treatment group largely got loans from microfinance institutions, followed by banks, while the control group primarily secured loans from banks, followed by microfinance institutions. Other common sources were self-help groups and the residual category of 'others', which included informal loans from moneylenders, family, friends, and acquaintances. The most frequently cited reasons among respondents for taking loans were for hospital expenses or health issues, marriages or other functions, construction or repair of houses, educational needs, and household expenses. Several women even talked about taking loans to pay off prior and existing debts.

WOMEN'S PAID AND UNPAID WORK

With the concerns expressed by feminists that cash transfer schemes would negatively impact women's paid employment (Khaitan, 2021; Tulsyan, 2021; TNN, 2022), we specifically asked both groups detailed questions that could account for the multi-faceted nature of their paid and unpaid work. Our questions were built on decades of efforts by feminist economists to craft a comprehensive understanding of women's work, both paid and unpaid (Luxton, 1997; Oxfam, 2018; International Labour Organization, 2021); including paid employment outside the home, paid work inside the home such as bidi rolling or tailoring, unpaid work that generates income, unpaid work that saves household expenditure and finally, unpaid domestic and care work.

PAID WORK

At the outset, 51% of the beneficiaries engaged in paid employment, compared to 39% of the control group. The occupational distribution of women from both groups engaged in paid employment is presented in Figures 23, 24.

> *"We don't have any jobs or source of income. We are all just coolies."*

FIGURE 23: WHETHER TREATMENT AND CONTROL RESPONDENTS HAD PAID WORK

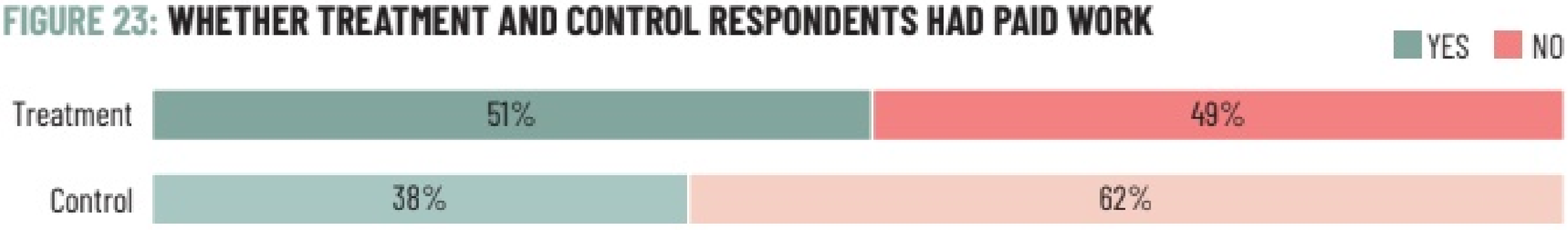
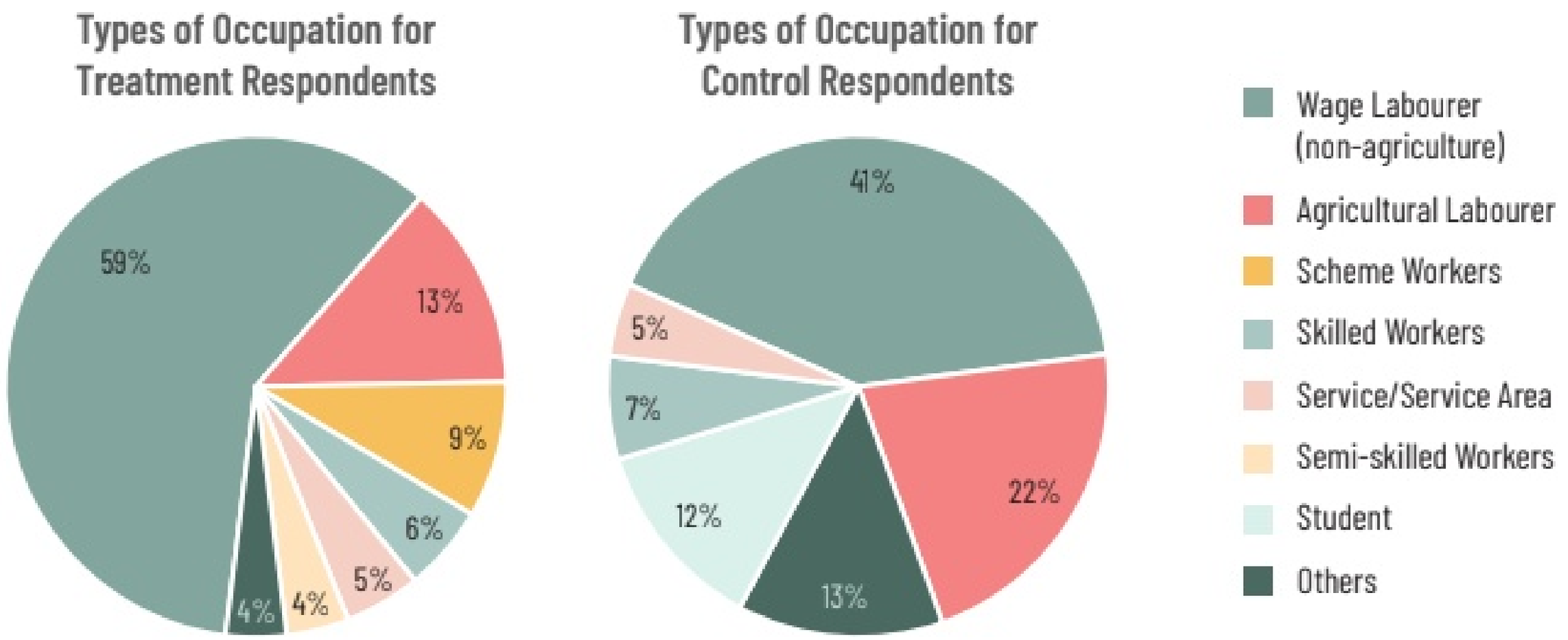


FIGURE 24: TYPE OF OCCUPATION AMONG RESPONDENTS WHO HAD PAID WORK



However, the impact of the scheme on paid and unpaid work is mixed. Beneficiaries are more likely to engage in paid work both inside and outside the home but earn less. They also tend to do less UDCW on the whole compared to the control group. We found that the beneficiaries are:

FIGURE 25: IMPACT OF KMUT AMOUNT ON PAID WORK OUTSIDE THE HOME

3 percentage points ↑ more likely to engage in paid work outside the home, but this difference is not statistically significant.

- Income from paid work outside the home ↓ decreases by **95 percent**

FIGURE 26: IMPACT OF KMUT AMOUNT ON PAID WORK INSIDE THE HOME

5 percentage points ↑ more likely to engage in paid work inside the home

- Income from paid work inside the home ↑ increases by **36 percent**, but this difference is not statistically significant.

FIGURE 27: IMPACT OF KMUT AMOUNT ON UNPAID WORK GENERATING INCOME

0.7 percentage points ↑ more likely to participate in unpaid work generating income, but this difference is not statistically significant.

- Savings from unpaid work ↑ increases by **14 percent**

Some women stated that they used the KMUT amount to travel to their paid work engagement. For example,

> *"With this 1000 rupees I am now able to tell my son that I can pay for the petrol, and he can drop me at the worksite."*

> *"I use the money to pay for an auto to get to 100 days' work."*

One woman used the money to generate paid work inside the house, stating:

> *"I don't have my husband. I used this money to spend for the children and saved some. Then I took a loan and bought a grinder. Now I use the grinder to grind batter and sell it. It helps me generate some income. I used it to buy the grinder, then for the business rotation. To buy raw materials and vessels and so on."*

Similarly, one woman used the KMUT amount to help make her unpaid domestic and care work easier:

> *"I saved the KMUT money and bought a washing machine because I found the manual washing to be very difficult. We do have a water shortage and using the washing machine to wash clothes has made my life much easier."*



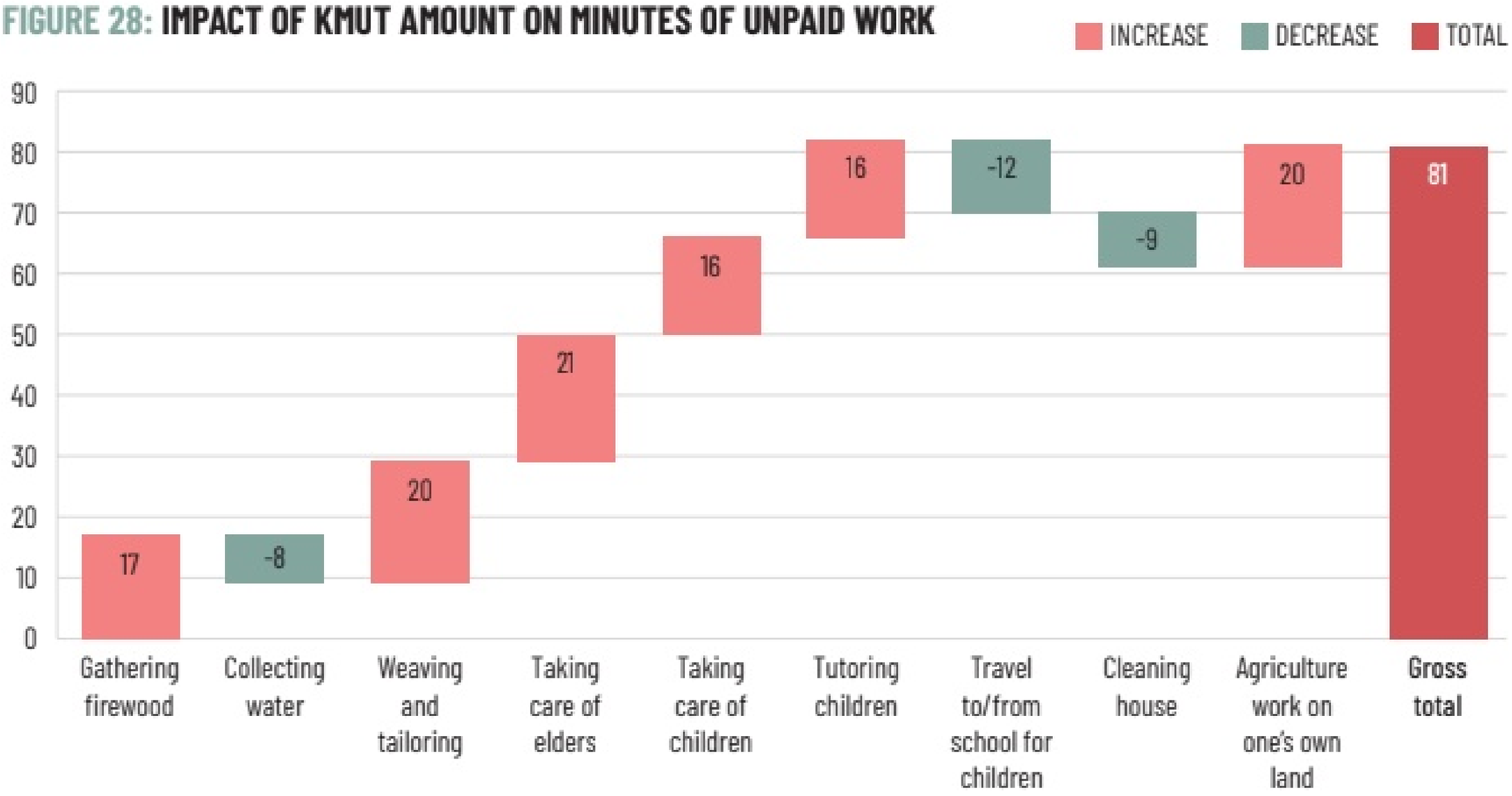
Photo 7: Survey Interview with Respondent

MINUTES OF UNPAID WORK

Following the large-scale survey, we undertook a smaller survey among 1228 respondents, with 488 in the control group and 740 in the treatment group, to document in detail time spent by both groups across 22 categories of unpaid work.[18] On average, beneficiaries spent 6.8 to 6.9 hours a day on unpaid work while the control group spent between 7.8 to 8.4 hours on such work.

Considering 9 categories of unpaid work, we find that compared to the control group, the beneficiaries spend 21 minutes more in taking care of elders, 20 minutes more in weaving and tailoring, 17 minutes more in gathering firewood, 16 minutes more in taking care of children, 16 minutes more in tutoring children, and 20 minutes more in doing agriculture work on one's own land. They spend 12 minutes less in travel time to drop off and pick up children from school, 9 minutes less in cleaning house, and 8 minutes less in gathering/collecting water.

FIGURE 28: IMPACT OF KMUT AMOUNT ON MINUTES OF UNPAID WORK



18 The 22 categories were: cooking; working in vegetable garden/orchard; gathering firewood; gathering water; collecting fruit, vegetables, fish, small game for consumption in household; weaving basket/mat; husking paddy; preparing jaggery; making cow dung cakes for fuel; tending to livestock (cows, goats, pigs); weaving /tailoring; caring for elderly; caring for children; tutoring children; travel time to drop off and pick up children from school; laundry/washing clothes; cleaning house; thatching mud house; caring for a person with disability; representing family at social functions; doing agricultural work on one's own land; any other unpaid work.

INTRA-HOUSEHOLD BARGAINING POWER

There were also positive shifts in intra-household bargaining power, with the beneficiaries having greater awareness and say in financial decisions as well as other aspects of their life including health and mobility. Beneficiaries had more family support in completing household tasks but there was no impact on family members' respect for and recognition of the beneficiaries' unpaid work. In particular:

FIGURE 29: **IMPACT OF KMUT AMOUNT ON AUTONOMY AND DECISION-MAKING**

Say in decisions regarding husband's money ↑ increases by **12 percentage points**

Husband's trust in wife with money ↑ increases by **2 percentage points**

CONFLICT AND VIOLENCE

MAJOR AND MINOR VIOLENCE

We surveyed the treatment and control group on the frequency of conflict (in general), the occurrence of various particular forms of physical and sexual violence over the past year, as well as the frequency of these varied kinds of violence. We found a decrease in daily conflict. However, we found statistically significant increases in particular kinds of violence as noted below:

FIGURE 30: **IMPACT OF KMUT AMOUNT ON CONFLICT**

Significant ↓ decrease in daily conflict by **23 percentage points** for beneficiaries

Among the women who stated they did experience some forms of minor violence; the scheme led to a shift in violence in the following ways:

FIGURE 31: **IMPACT OF KMUT AMOUNT ON CONFLICT AND MINOR VIOLENCE**

Husband's anger/jealousy when talking to other men ↑ increases by **8 percentage points**
[Affecting 171 Treatment respondents and 93 in Control]

Husband accusing wife of unfaithfulness ↑ increases by **6 percentage points**
[Affecting 88 Treatment respondents and 35 in Control]

Husband insisting on knowing wife's whereabouts ↑ increases by **7 percentage points**
[Affecting 84 Treatment respondents and 58 in Control]

Husband preventing wife from meeting a female friend ↑ increases by **4 percentage points**
[Affecting 69 Treatment respondents and 47 in Control]

- Frequency of meeting female friend ↓ decreases by **13 percentage points** over 12 months

Overall, it appears the reporting of violence has increased.

Among the women who stated they did experience some forms of major violence; the scheme led to a shift in violence in the following ways. We also note wives' infliction of violence on their husbands.

FIGURE 32: IMPACT OF KMUT AMOUNT ON MAJOR VIOLENCE

Frequency of husband pushing the wife ↑ increases by **34 percentage points** over 12 months
[Affecting 206 Treatment respondents and 114 in Control]

Frequency of husband twisting the wife's arm ↑ increases by **34 percentage points** over 12 months
[Affecting 160 Treatment respondents and 103 in Control]

Hitting husbands ↑ increases by **16 percentage points**
[Affecting 72 Treatment respondents and 80 in Control]

There was also an increase in some forms of sexual violence:

FIGURE 33: IMPACT OF KMUT AMOUNT ON SEXUAL VIOLENCE

Forceful sex ↑ increases by **4 percentage points** over 12 months
[Affecting 77 Treatment respondents and 37 in Control]

- Frequency of forceful sex ↑ increases by **38 percentage points** over 12 months

Forceful sexual acts ↑ increase by **3 percentage points**
[Affecting 30 Treatment respondents and 10 in Control]

- Frequency of forceful sexual acts ↑ increases by **53 percentage points** over 12 months

Abortions/miscarriages/unplanned pregnancies ↑ increase by **8 percentage points**
[Affecting 37 Treatment respondents and 13 in Control]

HUSBAND'S ALCOHOL CONSUMPTION

The survey also demonstrated an increase in alcohol consumption of the beneficiaries' husbands. In particular:

FIGURE 34: IMPACT OF KMUT AMOUNT ON HUSBAND'S ALCOHOL CONSUMPTION

The consumption of alcohol by husbands ↑ increases by **14 percentage points**
[Affecting 499 Treatment respondents and 302 in Control]

- There was no effect on the frequency of husbands' alcohol consumption over 12 months

But the frequency at which their husbands demand money for alcohol shows a ↓ decrease of **13 percentage points** over 12 months

CHAPTER 8

RECOMMENDATIONS

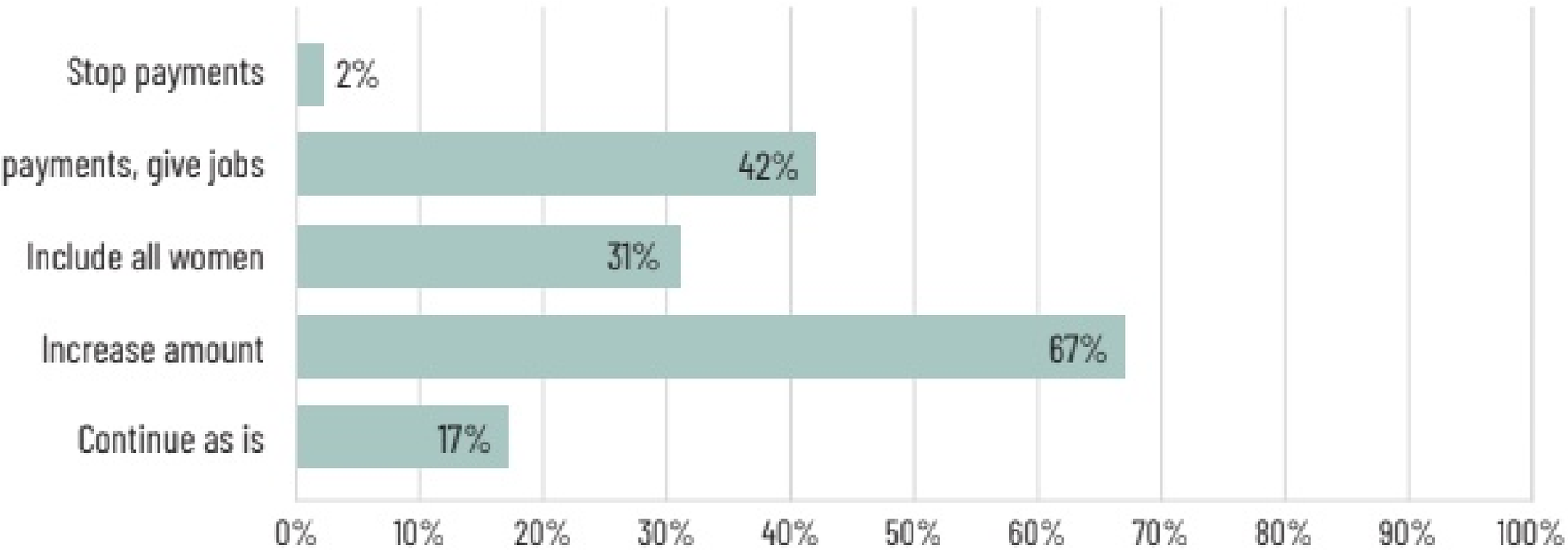
In our study we found that none of the feared negative impacts on women's education and paid employment have materialised. While the effects on food consumption, paid and unpaid work, and violence are mixed, a key finding of this study is that the KMUT scheme has had several positive first order effects, especially on women's financial inclusion, access to financial infrastructure, financial independence, and financial wellbeing. There are also important shifts in intra-household bargaining power, with women feeling a sense of autonomy and increased decision-making power. However, efforts must be made to realise the stated goals of the KMUT scheme, and for using it as a foundation to achieve women's empowerment.

RECOMMENDATIONS FROM THE BENEFICIARIES

» INCREASE AMOUNT UNDER AND EXTEND THE REACH OF THE SCHEME

We asked the respondent beneficiaries what their recommendations for the scheme were. The largest proportion at 67% felt that the amount should be increased, with 31% calling for the state to expand the scheme to all women in the state. 17% of the respondents felt that the scheme should be kept as is. Only 2% felt that the payments should be stopped completely, while 42% said that the scheme should be stopped and women should be given jobs instead.

FIGURE 35: **BENEFICIARIES' SUGGESTIONS FOR CHANGES TO KMUT SCHEME**



> *"When I fall ill, I am able to buy medicines at the pharmacy even when my husband is not at home, it would be better if this amount is more. It will make me independent financially and not expect from someone else. However, we are all coolies and it would be better if we have more."*

» RECONSIDER ELIGIBILITY CRITERIA

48% of the respondents from the treatment group stated that they were aware of the eligibility criteria. As a part of the survey, we then took the respondents through the eligibility and exclusion criteria under the scheme and asked them if they agreed with each of the requirements. We found that a minimum of 80% of respondents agreed with 8 out of 11 of the criteria we asked them about.[19] The only criteria which they disagreed with related to pension, with widow pension seeing the lowest level of agreement at 55%.[20]

The rationale for excluding women from KMUT on the basis that they or a household member got old age or widow pension was that the monthly pension levels would be increased, from Rs. 1000[21] to Rs. 1200[22] from August 2023 (Sivapriyan, 2023). Based on public pronouncements by the Chief Minister, the KMUT notification was then amended to state that a woman could apply for KMUT even if a household member received old age or widow pension (Government of Tamil Nadu, 2023b). In June 2025, the government further amended the criteria such that eligible women in households where another member of the household was receiving pension as a widow including as a deserted widow, deserted woman, a poor unmarried woman above the age of 50 years or a special time scale government employee could avail of KMUT (Government of Tamil Nadu, 2025). However, this still meant that the KMUT beneficiary herself cannot avail of both KMUT and the old age or widow pensions. This must be reconsidered in light of the particular vulnerabilities of women who receive such pensions, who would need the additional income from KMUT as well.



Photo 8: Post-survey FGD

19 Exclusion criteria and percent of respondents that agreed: annual income above 2.5 lakh - 82%; owning more than 5 acres of wet land or more than 10 acres of dry land - 84%; consuming more than 3600 units of electricity in a year - 80%; Receives government employee pension - 86%; family member is Government employee - 86%; family member is elected representative - 88%; paying IT/GST - 87%; having 4-wheeler for own use - 80%.

20 Exclusion criteria and percent of respondents that agreed: Received old age pension - 61%; received widow pension - 55%; receives unorganized workers welfare pension - 71%.

21 Approximately 12 USD.

22 Approximately 14 USD.

As stated above, 31% of the respondents said that the scheme should be expanded to include all women. In FGDs, beneficiaries highlighted that they wanted all women to receive the KMUT transfer because they had no access to their husband's income and as a matter of right for every woman:

> *"Even for a well-to-do married woman, the money she has might be the amount given by her husband. She might have all the amenities, but this 1000 rupees has to reach her too according to me. Because she might be a housewife. However rich her husband might be she will still have the yearning to have something for herself. Hence instead of the eligibility being based on the financial status the amount can be given based on the fact that she is a lady. She should also have a right to this money."*

> *"This should not be based on the financial situation of the beneficiary but she should be getting by virtue of being a woman. Let the old age pension beneficiaries also get this amount. Let the differently abled also get the KMUT. This amount is given as respect for the woman. Thereby everyone should get it."*

> *"Yes, I know of a man who uses a four-wheeler for income and he keeps back the income for himself, but what will she get? He doesn't even give the wife any money for household expenses. This amount would have been helpful for her right?"*

Beneficiaries spoke about relaxing other criteria relating to land ownership as well.

> *"Landowners cannot survive without the rains right. There are so many landowners who go through a difficult time during the droughts and do not even have money to feed their cows. We can relax that criteria."*

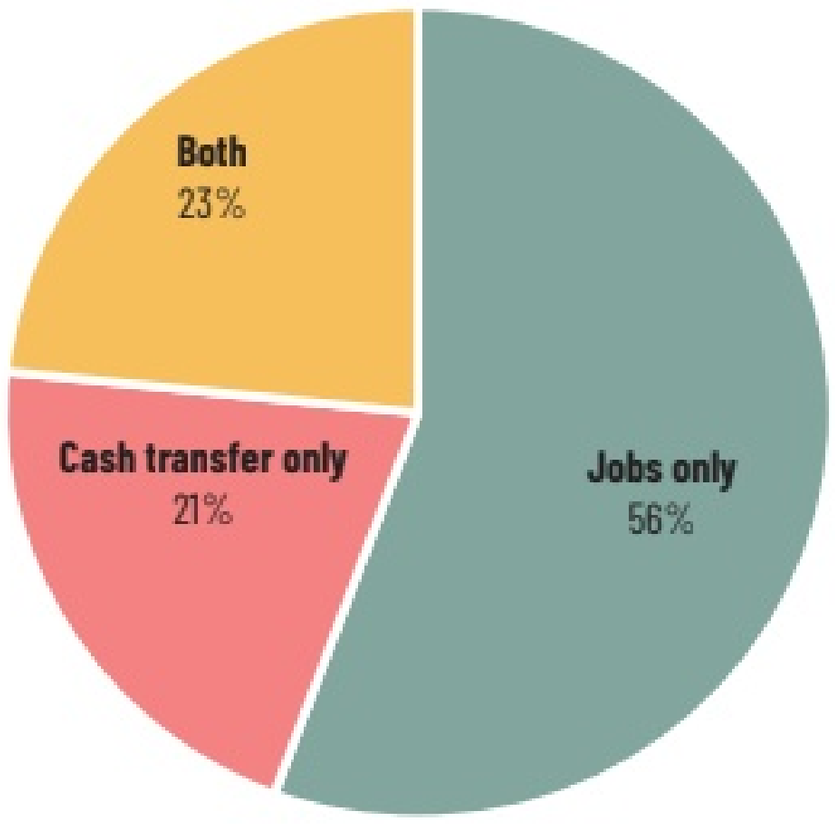
In a June 2025 notification, an otherwise eligible woman in a household owning a four-wheel vehicle obtained through government subsidies would not be excluded from availing KMUT. This is a welcome move. Further, 23% of the women knew someone who was eligible for the KMUT scheme, but who had either been excluded or was not availing of its benefits. While the reason for such exclusion is not available from the survey data, it does indicate the need to expand the reach of the scheme. During FGDs, the beneficiaries emphasised the need to provide support in compiling the necessary documentation to access the KMUT scheme in order to ensure that those eligible for the scheme derive its benefit. Applicants must also receive clear reasons for exclusion, alongside accessible and responsive grievance redressal mechanisms for them to appeal such rejections.

» WOMEN, IN FACT, PREFER PAID WORK

Critics of cash transfer schemes argue that schemes like KMUT will discourage women from taking up paid employment, which is a concern given India's low female labour force participation rate (Singh and Pattanaik, 2020; Khaitan, 2021; Tulsyan, 2021; TNN, 2022; Deshpande and Kabeer, 2024). Hence, we asked beneficiaries what they preferred: paid work or a cash transfer scheme. 56% of the respondents said they would prefer to access paid work instead of the cash transfer scheme, with only 21% saying they preferred receiving just the cash transfer. The remaining 23% said it would be better to receive both paid work and the cash transfer.

Thus, cash transfers are not discouraging women's desire for paid work. This finding is supplemented by an identified causal increase in paid work among beneficiaries compared to the control group. We also found that the 21% who chose cash transfers were often unable to take up paid work due their advanced age, medical conditions, or childcare responsibilities.

FIGURE 36: BENEFICIARIES' PERCEPTION OF KMUT SCHEME VERSUS JOBS



> *"Instead of this 1,000 rupees job opportunities can be provided. But don't stop this 1,000 rupees for that. Some might say that they don't need this money. Even yesterday because I had this money, I was able to get fruit for 250 rupees for my daughter. I am able to afford this because of this cash. I can't ask my husband to spend for the fruit. This is a good scheme by the CM and hence it need not be stopped, but everyone could be benefitted from this scheme."*

RECOMMENDATIONS FOR A GENDER TRANSFORMATIVE WELFARE REGIME

» DELIVER FINANCIAL LITERACY MODULES ON LOANS, SAVINGS, INVESTMENTS THROUGH MOBILE PHONES AND IMPROVE ADULT LITERACY THROUGH RURAL LIBRARIES

The data from the study also revealed potential routes to improve women's financial literacy and access to financial infrastructure in the state. For example, 67% of the beneficiaries we met had their own phones, of which half had smartphones. Given the high levels of debt and women's lack of knowledge of savings and investments, there is an urgent need to meaningfully improve financial literacy. A potential low-cost intervention could be creating modules on loans, savings, and investments delivered to women through mobile phones, which could be widely effective in improving women's overall financial awareness and wellbeing. However, as only two-thirds of the women could read the SMS that they received, any such efforts would necessarily have to be accompanied by measures to improve adult literacy through rural libraries, modelled along the lines of the Gram Panchayat Library and Information Centres in Karnataka (Government of Karnataka, 2025).

» UNDERTAKE CARE AUDIT OF WELFARE REGIME; EMBED KMUT IN RIGHT TO CARE LAW

While the KMUT represents a valuable step forward in expanding Tamil Nadu's pre-existing and robust welfare regime, concerns remain about the design and implementation of this wider welfare regime, especially from a feminist perspective. For instance, a widow cannot avail of widow pension if she is below the age of 40 or if she has an adult son. The presumption here is that she should not rely on the state when there are other men that she can rely on, that is, her male children or a husband by getting remarried. To address these issues, a gender transformative audit of various welfare schemes is essential.

The KMUT scheme must also be embedded within a right to care law to help ensure its long-term sustainability and impact, alongside appropriate messaging that can help facilitate the empowerment of women. We can look to Latin American countries for guidance; wherein model legislations have been developed that integrate a right to care within the legal framework (Inter-American Commission of Women, 2022; García and Vaeza, 2023). This assumes significance in light of the recognition by the Supreme Court of India that women's UDCW is on par with a paid occupation (Kotiswaran, 2021).

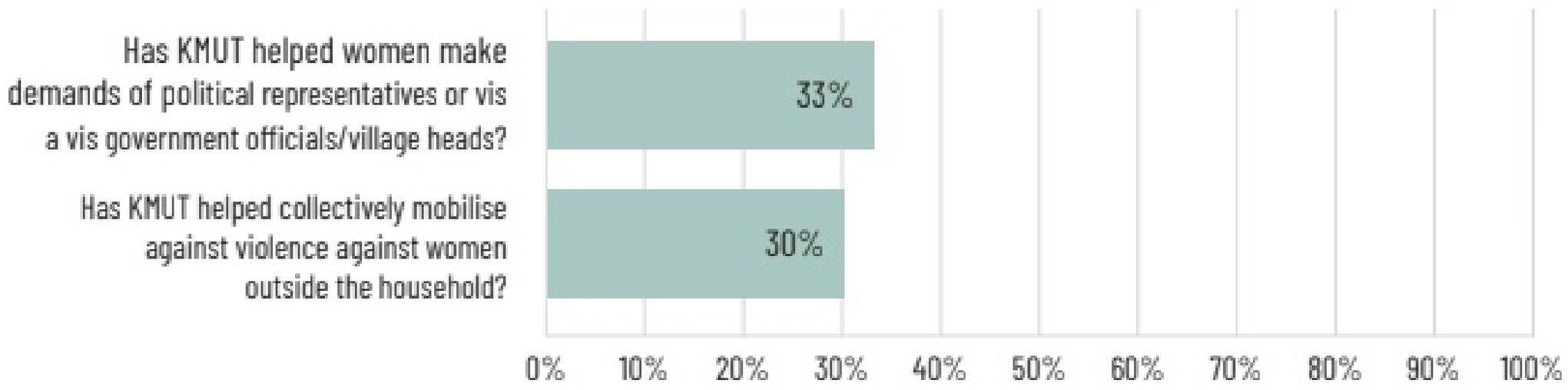
» INVOLVE GENDER EXPERTS AS STAKEHOLDERS FOR REGULAR EVALUATIONS OF KMUT

To ensure the long-term success of the KMUT scheme, there must also be provision for its periodic evaluation. These must be conducted in consultation with the beneficiaries themselves as well as with women's groups. In Latin America, government's lack of consultation with women's groups meant that conditional cash transfer schemes became increasingly onerous for women and eventually undermined empowerment effects (Molyneux and Thomson, 2011). The KMUT scheme could avoid a similar fate by involving independent gender experts, women's groups, and the beneficiaries themselves in its evaluation and implementation.

» AMPLIFY EMPOWERMENT EFFECTS OF KMUT

The KMUT scheme already shows some empowerment effects. A third of the women surveyed said they were able to make demands of political representatives and officials and that the scheme helped them mobilise against gender-based violence outside the household. These effects can be amplified through further support of KMUT beneficiaries and sensitisation of the public.

FIGURE 37: BENEFICIARIES' PERCEPTION OF KMUT SCHEME'S IMPACT ON THEIR MOBILISATION



» STRONGER MESSAGING ON RECOGNITION OF UDCW ALONGSIDE MEASURES FOR REDUCTION AND REDISTRIBUTION OF UDCW

Tamil Nadu has played a pioneering role in implementing SDG 5.4 by conceptualising the KMUT cash transfer as offering recognition of women's UDCW. However, beneficiaries themselves are not aware of this essential aspect of the scheme. Not only should there be stronger messaging on how KMUT recognises women's UDCW, the state must also make concerted efforts through publicity campaigns and school education programs to sensitise men, girls and boys towards the reduction and redistribution of UDCW in accordance with SDG 5.4.

In FGDs, women discussed how even their children would ask them to bring water or refuse to keep their plates in the sink for washing. Educating and sensitising children and young adults on the UDCW performed by women in the household is essential to achieve a shift in social norms. An example of such efforts is

the training provided by the 'Time to Care' toolkit developed by Flow India (2025) under the Laws of Social Reproduction project for 1000 middle school students around the country.

The government should also consider efforts to reduce UDCW. For example, in 2021 when the Left Democratic Front won the elections in Kerala, they introduced a Smart Kitchen scheme that would provide subsidised credit for purchasing kitchen appliances, which could help lessen the drudgery of household work (Explained Desk, 2021).[23] The care burdens of women in rural areas could also be reduced by investing in care infrastructure along the lines of the *Koosina Mane* scheme in Karnataka, which provides creches for children of NREGA workers between the ages of 0-3 (The Hindu Bureau, 2024).

However, norms around social reproduction are deeply intertwined with social hierarchies and therefore any reforms need to be designed with specific contexts in mind. When we posed the possibilities of having communal community kitchens and community laundromats in FGDs, respondents cited caste barriers as a significant obstacle. Thus, the socialisation of care is not a straightforward solution and must be reimagined for the Indian context.

CHANGES IN STRUCTURAL CONDITIONS FOR WOMEN'S EMPOWERMENT

During the FGDs, two seemingly unrelated issues that significantly undermine women's financial condition arose. One pertains to the predatory practices of microfinance institutions and the other to the lack of a robust public health infrastructure. We address both in turn.

» REGULATE MFIS TO ENSURE ETHICAL LENDING PRACTICES

During the FGDs, a recurring theme that emerged was the burden of substantial loans from MFIs that engaged in predatory practices. In one village in particular, women noted:

> *"We are unable to eat or socialize, we become embarrassed and withdrawn, we feel terribly pressured and obligated to pay back [the loan]. We feel as if the time is running [out], we have no energy, and the pressure is getting intense. The problem is that we don't have jobs and no proper income for the amount of efforts we take for the job."*

To address this concern, and ensure women's overall financial wellbeing, the government must enforce the *Tamil Nadu Prohibition of Charging Exorbitant Interest Act, 2003*. We also recommend the setting of clear limits on interest rates and fees that MFIs can charge to borrowers, which will help prevent usury and exploitative lending practices. Furthermore, the government should mandate that MFIs disclose the total cost of the loan – including interest rates, fees, and any other charges –to all borrowers in a transparent and easy-to-understand manner, before they commit to a loan. There should also be an independent grievance redressal mechanism for fair resolution of complaints.

The recent decision of the government to introduce the Tamil Nadu (Prevention of Coercive Action) Bill, 2025 is a welcome move in this direction (The Hindu Bureau, 2025b).

23 This was eventually replaced by the 'Easy Kitchen' project providing financial assistance from local bodies to women belonging to socio-economically vulnerable families to renovate kitchens to have basic facilities such as sinks, cupboards, and water tanks. See Web Desk, 2024.

» IMPROVE PUBLIC HEALTH INFRASTRUCTURE AND HEALTH MESSAGING

Beneficiaries mentioned a range of health issues in our FGDs including inadequate and corrupt health infrastructure, high medical expenses, as well as the excessive alcohol consumption by men, which led to conflict, violence, and abuse.

PUBLIC HEALTH INFRASTRUCTURE

Many women spent the KMUT cash transfer amount on medical expenses, revealing gaps in the public healthcare systems. Efforts must be made to address these gaps so that low-income families can access affordable, high-quality, healthcare services without having to depend excessively on out-of-pocket expenses or be driven into debt because of medical needs. Rampant corruption in the public health sector must also be addressed.

> *"They will ask for 100 rupees even to put the newborn on the bed. They will ask 100 rupees if it is a girl child and 1000 rupees if it is a boy."*

> *"They will put us in a "thai sei" (maternity and child welfare) vehicle once discharged and they will ask for cash for that too. The driver will ask for almost 200 to 300 rupees (to drop the mom and baby home). That vehicle is supposed to be free."*

ALCOHOL

Significantly, 96% of the 2221 women we surveyed across both the treatment and control groups want the Tamil Nadu State Marketing Corporation Limited (TASMAC) shops to be closed. These TASMAC shops are owned by the government and exclusively sell alcohol in the state.

> *"The government says that the income from the alcohol shops only fund all the welfare measures taken by the State. The men also say that the income comes from the liquor. People in fact say that the illicit liquor shops need to be closed and not the government approved TASMAC shops that generate income for the State. When we go for protests, there are chances that we will be hacked to death and thrown to pieces in the woods."*

Speaking of the societal impact of alcoholism, one woman noted:

> *"Everything collapses if there is such a problem. Everyone becomes affected because of addiction. The husband, the wife and the children are all affected because of this single problem. My sister is affected because of her husband's addiction. My parents are of support, and she is able to survive because of this support. That too because of the fact that my parents are within the same village. I have lived close to her house, and I could see the husband being intoxicated whenever he comes home. He will in fact torture the kids. She had to work and save the family."*

Despite the call for shutting down of TASMAC shops, the complete prohibition of alcohol carries the danger of illicit liquor markets, which in turn pose serious risks to the health and safety of consumers (Mishra, 2024;

Bali and Grounds, 2025). However, prohibition could also reduce violence against women (Chakrabarti *et al.*, 2024), a concern that did emerge in the course of this study (see Conflict and Violence). At the very least, we recommend efforts to monitor the minimum drinking age, to package alcohol in ways that incentivize responsible drinking practices, and public service advertisements on the harms of drinking.

> *"There are so many unmarried girls because we don't find suitable grooms. So many pregnant young women who lose their husbands to alcohol. The men come home drunk and create so many issues at their houses. They create a ruckus and then they storm out, drink again and not come back home but sleep somewhere else"*

In addition, in order to prevent violence against women, the state must establish safe spaces and helplines for women in the state who face violence and abuse.

HEALTHY DIET

The results of the survey show that beneficiaries are likely to eat more fish and eggs, but also less nutrient dense foods. While a change in the scheme design (to one of conditional nature) is not necessary or desirable, the government must consider messaging to the general public on the benefits and necessity of a healthy diet. The government should reconsider items given out under the Public Distribution System to enhance food diversity (Tamil Nadu Civil Supplies Corporation, 2020).

CHAPTER 9

CONCLUSION

The KMUT scheme has emerged as a success story in many ways. For one, it has been efficiently and effectively implemented. The scheme has shown varied positive first order effects, particularly on the empowerment and financial independence of the women beneficiaries. We suggest that efforts be made to improve the scheme's implementation and reach. However, the KMUT is only a single, but important, step forward in efforts that must be taken to empower women across the state.

There must be concerted efforts to address the broader vision of social justice under the Dravidian model of development. This must include ensuring effective access to education, health, and financial infrastructure, so that the benefits of the scheme can be fully realised. Furthermore, the KMUT's specific emphasis on recognition of UDCW must also inspire efforts to redistribute UDCW under the United Nation's SDG 5.4. The beneficiaries themselves as well women's groups must be involved in the design and improvement of this broader welfare regime alongside the recognition and implementation of a right to care. It is only then that unconditional cash transfers can become gender transformative (Kotiswaran *et al.*, 2025).



Photo 9: Pre-Survey FGD

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